

Accountability to affected people Handbook on implementation in emergency response



World Health
Organization

Accountability to affected people

Handbook on implementation in emergency response

Accountability to affected people: Handbook on implementation in emergency response

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Foreword

“The right to participate is an essential feature of the right to the highest attainable standard of health. People have the right and duty to participate individually and collectively in the planning and implementation of their health and wellbeing.” (1)

Dr Tedros Adhanom Ghebreyesus, Director-General, World Health Organization

As of December 2024, the World Health Organization (WHO) was responding to 17 acute and protected emergencies requiring the highest level of support (G3 and P3), out of a total of 42 emergencies that the organization was responding to. It is estimated that 305 million people will require humanitarian assistance in 2025 (2). In an ever-changing world fraught with complex emergencies, WHO’s role in responding to health crises has never been more critical. At the heart of these efforts lie the individuals, communities and populations directly impacted by such emergencies. Affected communities are the experts on their health needs and priorities. They are the first to respond to family and community needs when a crisis hits. They understand what works and what does not work. To deliver effective humanitarian health responses, WHO must also harness and act on this knowledge and capacity, in conjunction with evidence and scientific facts, and put people affected by crisis at the centre of our responses.

Implementing humanitarian responses that are accountable to affected people means empowering people with information, opportunities to express their needs and to take an active part in decision-making. It is about being inclusive and encouraging the participation of the most vulnerable in communities and contributes considerably to community health protection. Most importantly it is about WHO being answerable to affected people by responding to their queries and feedback, informing them of progress and setbacks, and seeking out and acting on their ideas.

This approach is embedded in WHO’s constitution (3), which outlines that “Informed opinion and active co-operation on the part of the public are of the utmost importance in the improvement of the health of the people.” They are also a key feature of the community protection component in the health emergency prevention, preparedness, response and resilience (HEPR) framework (4), which states that “communities must be at the centre of efforts to prepare for, prevent and respond to health emergencies. ... interventions must be co-created with affected communities”.

Prioritizing accountability to affected people (AAP) in WHO’s emergency responses is not only an ethical imperative, but also a strategic choice that will lead to more effective interventions and safer and better protected communities throughout the emergency management cycle. Principals have committed to it, and WHO is determined to further operationalize this approach in emergency response operations under its *Emergency response framework*, edition 2.1 (5). The benefits of implementing such an approach are clear, and the time to scale up action is now. As we strive to build a healthier world, let us ensure that the voices of those we serve remain at the centre of our efforts.



Dr Mike Ryan

Executive Director Health Emergencies Programme/Deputy Director General WHO

A handwritten signature in blue ink, appearing to read 'Mike Ryan'.

Acknowledgements

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How the Handbook was developed

This is the first WHO guidance that supports the integration of AAP in WHO's Health Emergencies Programme (WHE) operations. It should be used together with the Global Health Cluster ***AAP – operational guidance (6)*** and ***Minimum standards – AAP (7)*** which guide health partners in the implementation of AAP.

The following process was followed:

- desk review of existing and upcoming global commitments and standards that relate to AAP as well as other agency-specific guidance on implementing AAP;
- key informant interviews with knowledgeable stakeholders and end users within WHO;
- presentation of draft Handbook to WHO headquarters, regional and country-level audiences for internal peer review; and
- external peer review of the Handbook.

Abbreviations

AAP	accountability to affected people
CFM	complaints and feedback mechanism
CHS	Core Humanitarian Standard
ERF	WHO emergency response framework
FAQ	frequently asked question
HEPR	health emergency preparedness, response and resilience
HH	household
IASC	Inter-Agency Standing Committee
JOR	joint operational review
LGBTIQ+	lesbian, gay, bisexual, transgender, intersex or queer
NGO	nongovernmental organization
PHSA	public health situation analysis
PSEA	protection from sexual exploitation and abuse
PRSEAH	prevention and response to sexual exploitation, abuse and harassment
SEA	sexual exploitation and abuse
SMS	short messaging service
SOP	standard operating procedure
UN	United Nations
UNICEF	United Nations Children's Fund
WHE	World Health Organization Health Emergencies Programme
WHO	World Health Organization

Glossary

Accountability to affected people (AAP)

An active commitment to use power responsibly by taking account of, giving account to, and being held to account by the people humanitarian organizations seek to assist.

Affected people

Affected people are all the people in a context where their lives are directly or indirectly impacted by conflict, disaster or crisis.

Complaint

A complaint is a piece of feedback from someone who is objecting to the actions of an organization or its staff because they are unfair, unacceptable or not meeting expected standards and commitments.

Community engagement

The collaborative process that involves people in assessing their needs and in developing programmes that are acceptable and workable for them. The goal is to empower communities to develop shared leadership throughout the emergency response cycle.

Diversity

Diversity is the range of human differences, including gender, gender identity, age, nationality, ethnic background, beliefs, sexual orientation, ability or other characteristics. These characteristics will lead people to have different experiences, needs and vulnerabilities in humanitarian contexts.

Feedback

Feedback is a positive, negative or neutral opinion shared with an organization regarding its programmes, way of operating or the behaviour of its staff. Organizations may solicit feedback by proactively asking people for their opinions, or receive unsolicited feedback when someone chooses to approach the organization and provide their opinion.

Inclusion

Inclusion is the practice of providing equal access to opportunities, resources and rights for people who might otherwise be marginalized or excluded, such as minority groups.

Participation

Participation is people's active involvement and engagement in the decision-making and activities of humanitarian organizations. It is important that all people have equal access to participate, so that their views and opinions are reflected in decision-making.

Protection from sexual exploitation and abuse (PSEA)

PSEA is the package of measures taken by organizations to prevent and respond to sexual exploitation and abuse (SEA) of affected people by humanitarian staff. These measures aim to ensure affected people understand the expected conduct of staff and have access to safe and confidential reporting channels. SEA allegations are investigated and acted upon by organizations, and SEA survivors are provided with the required support.

Risk communication

Risk communication is the real-time exchange of information, advice and opinions between experts and officials and people who are facing a threat to their health, survival, or economic or social well-being. The goal of risk communication is to help people make informed decisions and take protective actions to mitigate the effects of the threat.

Sexual exploitation and abuse (SEA)

Sexual exploitation is any actual or attempted abuse of a position of vulnerability, differential power or trust for sexual purposes. It includes, but is not limited to, profiting monetarily, socially or politically from the sexual exploitation of another. Sexual abuse is the actual or threatened physical intrusion of a sexual nature, whether by force or under unequal or coercive conditions. SEA by humanitarian staff against affected people they serve represents one of the most serious breaches of accountability.

Chapter 1

Introduction

1.1 About this Handbook

1.1.1 Purpose

This Handbook was designed to support WHO Health Emergencies Programme (WHE) staff to put people affected by humanitarian crises at the centre of decision-making on emergency health responses, and to guide WHE staff in operationalizing WHO's commitments to accountability to affected people (AAP). This will enable WHO to meet the needs of affected people in a way that makes sense for them, that utilizes their existing capacities and strengthens their resilience in the long term.

By making sure that affected communities play an active role in decisions that affect the provision of humanitarian health assistance, we are ensuring higher quality and more effective outcomes that preserve the dignity of those we serve.

1.1.2 Audience

This Handbook is for country office, regional and headquarters WHE staff and other people who support country operations. The Handbook details AAP responsibilities across the emergency response framework (ERF) critical functions. All WHE teams must refer to it at all stages of the programme cycle and when drafting key response documentation.

1.1.3 Use

The Handbook is divided into eight chapters:

- Chapter 1 Introduction: overview of the Handbook and AAP, WHO's global AAP commitments and community engagement;
- Chapter 2 WHO's AAP principles;
- Chapter 3 AAP in WHO's emergency response framework: AAP responsibilities across the seven critical functions, signposts to relevant Handbook sections, and progress indicators;
- Chapter 4 AAP in the humanitarian programme cycle: minimum actions to integrate AAP across the programme cycle;
- Chapter 5 Participation and inclusion: minimum actions to ensure participation and inclusion of affected people in decision-making;
- Chapter 6 Information provision and transparency: minimum actions in providing adequate information and ensuring transparency for affected people;
- Chapter 7 Complaints and feedback: minimum actions to ensure that WHO receives and responds to feedback from affected people, and programmes are adapted accordingly; and
- Chapter 8 Partnerships, collective AAP and coordination: considerations when implementing AAP through and with government and nongovernmental organization (NGO) partners.

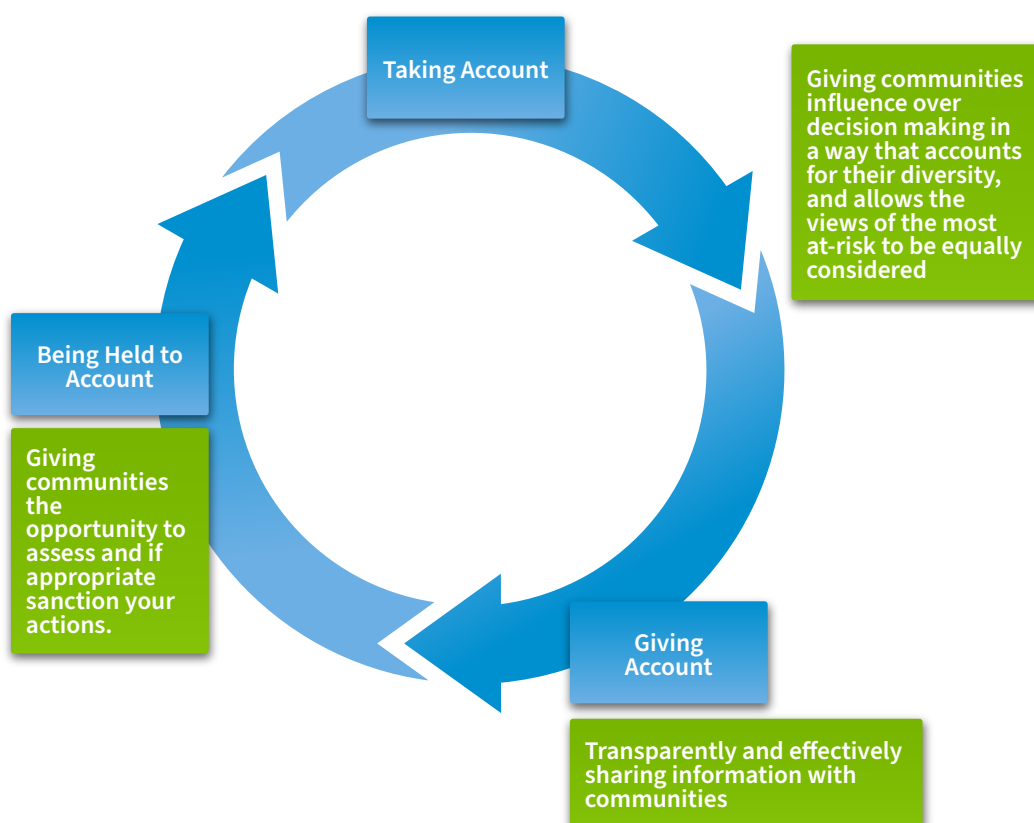
Resources are highlighted throughout the Handbook. The 11 annexes contain example templates, terms of reference, and content for needs assessments, operational plans, logframes, donor proposals and more.

This Handbook should be used to inform acute and protracted emergency response programmes directly implemented by WHO teams in country, through partnership with the health ministry, the health cluster and/or other implementing partners. Although this Handbook is aimed at WHO emergency operations, the concepts and ideas within are relevant across the humanitarian–development–peace nexus and should be used to influence all WHO programming.

1.2 What is AAP?

The Inter-Agency Standing Committee (IASC) defines AAP as “an active commitment to use power responsibly by taking account of, giving account to, and being held to account by the people humanitarian organisations seek to assist.” (8)

Fig. 1 AAP: a brief overview



Source: IASC (8)

1.2.1 A rights-based approach

AAP is founded on a rights-based approach that values affected people as rights holders who should be empowered to know and claim their rights and participate in decisions that impact their lives. WHO is a duty bearer. In implementing its AAP commitments, WHO increases its accountability for respecting, protecting and fulfilling the rights of affected people.

All WHO Member States have ratified at least one international human rights treaty that includes the right to the highest attainable standard of health (9). To deliver on these commitments, countries must mainstream fundamental human rights principles and standards in public health emergency preparation and response. These principles include:

- **non-discrimination and equality:** prioritizing the needs of those furthest behind to achieve equity;
- **participation:** empowering health service users and communities to engage in planning, decision-making and implementation processes for health across the programme cycle and at all levels of the system; and
- **accountability:** the requirement for countries to establish accessible and effective accountability (10).

Actively implementing these principles is key to achieving humanitarian health responses that are accountable to affected people.

1.3 Global commitments on AAP

The humanitarian sector has developed global commitments to assist agencies in ensuring their programming is more effective and accountable.

1.3.1 IASC

The IASC is the highest level humanitarian coordination forum of the United Nations system. It brings together the executive heads of 19 organizations and consortia, including WHO, to formulate policy, set strategic priorities and mobilize resources in response to humanitarian crises. In November 2017, the IASC Principals endorsed the following four Commitments (11) on AAP and protection from sexual exploitation and abuse (PSEA) (Fig. 2).

Fig. 2 IASC revised commitments on AAP and PSEA

Leadership	Demonstrate their commitment to Accountability to Affected Populations (AAP) and Protection from Sexual Exploitation and Abuse (PSEA) by enforcing, institutionalising and integrating AAP approaches in the Humanitarian Program Cycle and strategic planning processes, at country level and by establishing appropriate management systems to solicit, hear and act upon the voices and priorities of affected people in a coordinated manner, including for SEA, before, during and after an emergency.
Participation and partnership	Adopt agency mechanisms that feed into and support collective/coordinated people-centred approaches that enable women, girls, boys, men, including the most marginalised and at-risk people among affected communities, to participate in and play an active role in decisions that will impact their lives, well-being, dignity and protection. Adopt and sustain equitable partnerships with local actors to build upon their long-term relationships and trust with communities.

Information, feedback, and action	Adopt agency mechanisms that feed into and support collective and participatory approaches that inform and listen to communities, address feedback and lead to corrective action. Establish and support the implementation of appropriate mechanisms for reporting and handling of SEA-related complaints. Plan, design and manage protection and assistance programmes that are responsive to the diversity and expressed views of affected communities.
Results	Measure AAP and PSEA related results at the agency and collective level, including through standards such as the Core Humanitarian Standard and the Minimum Operating Standards on PSEA; the Best Practice Guide to establish Inter-Agency Community-Based Complaint Mechanisms (CBCM) and its accompanying Standard Operating Procedures.

Source: IASC (11)

In 2022 the IASC Principals released a statement affirming their commitment to AAP as central to principled humanitarian action. It pledged to support humanitarian coordinators, humanitarian country teams, clusters and individual agencies in prioritizing the implementation of AAP commitments in all humanitarian operations (12).

In 2023 the IASC Principals approved the *IASC collective AAP framework (13)* which enables resident/humanitarian coordinators, humanitarian country teams and UN country teams to prioritize actions to strengthen response-wide AAP across six outcome areas (preparedness, needs assessment, planning, funding, implementation and monitoring, evaluation and review). WHO must design and implement AAP activities in a way that contributes to collective AAP at interagency level. This is discussed more in Chapter 8 of this Handbook.

1.3.2 Sphere and the Core Humanitarian Standard

The Sphere handbook (14) health chapter outlines that “in applying the Minimum Standards, all nine commitments in the Core Humanitarian Standard should be respected. Feedback mechanisms must be put in place during healthcare responses” (pg295). The Core Humanitarian Standard (CHS) on quality and accountability (15) promotes “equitable and collaborative relations between people and communities and those working to support them. It aims to address power imbalances” (pg2). AAP is at the heart of what the CHS aims to achieve.

Fig. 3 Core Humanitarian Standards



Source: CHS Alliance (15)

1.3.3 Grand Bargain

The Grand Bargain 2016 (16) committed donors and humanitarian organizations to better humanitarian outcomes for affected people through enhanced efficiency, effectiveness and greater accountability.

This included a commitment to a ‘participation revolution’ to include people receiving aid in making decisions which affect their lives. Within the overall objective of the Grand Bargain 3.0 (17), endorsed in June 2023, three focus areas are envisioned to continue efforts to deliver on the commitments (namely participatory revolution, localization and quality funding) with more focused attention on country-level progress.

1.3.4 Localization

By shifting power and resources to local actors, localization enables affected populations to play a more active role in decision-making processes and allows humanitarian actors to draw on local knowledge, expertise and networks. WHO is in the process of finalizing a localization strategy to reinforce its principal mission in fragile, conflict-affected, vulnerable and protracted emergency settings: strengthening the capacity of national and subnational health authorities to prepare for, respond to and prevent health emergencies. This will also support WHO in implementing the Grand Bargain commitment (17) to “Provide greater funding and support for the leadership, delivery and capacity of local responders.” Achieving a more localized approach to humanitarian action is essential in delivering on AAP. It enables sustainable and accountable aid that addresses the needs and respects the values of affected people.

1.3.5 Humanitarian–development–peace nexus

Although this Handbook focuses on WHO health emergencies, it is important to note that affected people do not differentiate between humanitarian, development and peace organizations and programmes. Participation of communities in decision-making in development and peace programmes is as important. It has been ongoing for many years through social accountability approaches and participatory rural appraisal, among other approaches. The ideas and principles in this Handbook can and should also be integrated in development and peace programmes.

1.4 WHO and AAP

The values that underpin AAP are strongly reflected in WHO's constitution (3): "The informed opinion and active cooperation on the part of the public are of the utmost importance in the improvement of the health of the people."

These values have continued as central to the way that WHO works, and they feature in core response frameworks and approaches.

1.4.1 HEPR framework

This Handbook is aligned with and contributes to the WHO HEPR framework (4), in particular the community protection subsystem. This aims to deliver population and environmental health interventions intended to manage events in ways that are acceptable, relevant and meaningful to those at risk and do not inadvertently create harm. It does this by promoting a community-centred approach, harnessing local knowledge and ensuring health strategies are responsive to community input and feedback regarding their design, implementation, monitoring and evaluation.

1.4.2 Risk communication and community engagement

Risk communication is a key approach used by WHO to exchange information between experts and people who face risks to their health, to enable them to make informed decisions to mitigate these risks. Community engagement is the process of developing relationships with communities to better understand their needs and create responsive solutions that are acceptable and workable for affected people. This Handbook will support the implementation of this community engagement approach, ensuring that we involve communities in decisions that affect them.

1.5 Further reading

WHO competency framework, risk communication and community engagement: for stronger and more inclusive health emergency programmes. Geneva: World Health Organization; 2024 (<https://iris.who.int/handle/10665/376653>).

Chapter 2

WHO's AAP principles

The following principles should underpin all of WHO's work on AAP.

Safety and dignity

A 'do no harm' approach should be applied to the design and implementation of all programmes and AAP components. Risk factors for different groups should be considered, mitigated and monitored to make sure that WHO keeps affected people safe. Preserving and promoting the dignity of affected people is at the heart of AAP. This is achieved by listening to and responding to their needs and allowing their voices, concerns and capacities to be the main drivers of the provision of emergency health assistance.

Confidentiality and data protection

Confidential treatment of personal and sensitive information received from affected people is an important part of staff accountability in the implementation of AAP activities. Personal data should be stored securely and in a way that does not put people's dignity, safety or privacy at risk. Affected people need to give informed consent before giving humanitarian responders their personal information. Informed consent means they understand why their information is being collected, how it will be used, their right to confidentiality and their right to have their data deleted if requested.

Accessibility and non-discrimination

AAP activities should allow for the meaningful inclusion of affected people, in all their diversity, and support non-discrimination. This means that factors such as gender, age, disability, sexuality, religion and any other diversity factors that are relevant to the context should be considered and planned for to ensure that all affected people can provide feedback, participate in decision-making processes and benefit from health assistance on an equal footing.

Gender equality

Gender influences people's experience of and access to health care. Women and girls often face greater barriers than men and boys to accessing health information and services. AAP activities should prioritize hearing and responding to the voices of women and girls and designing AAP activities and mechanisms that are accessible to them.

Conflict sensitivity

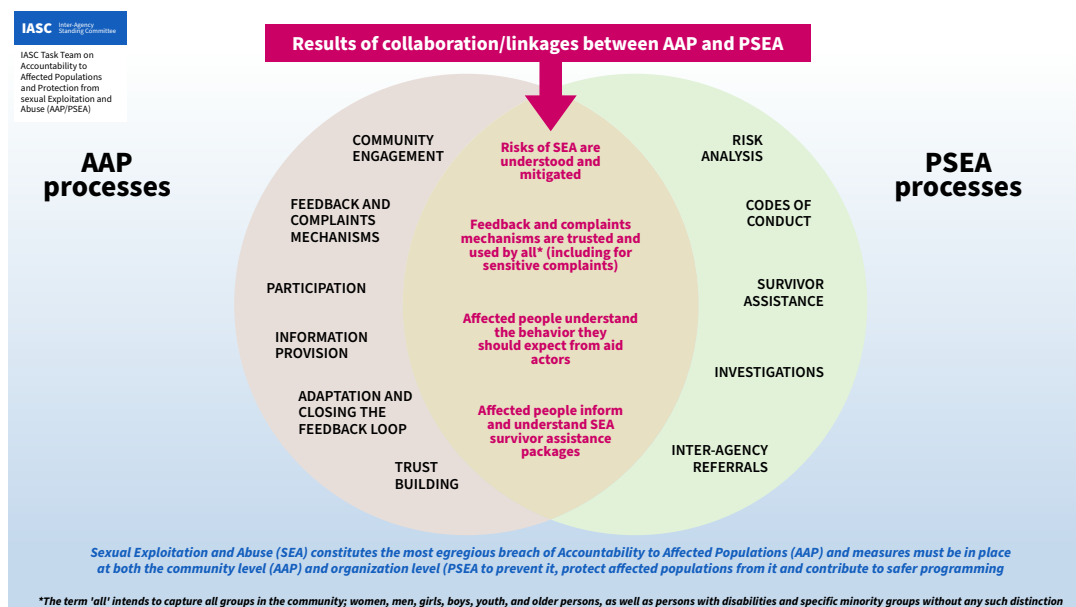
Given that the majority of WHO's emergency work takes place in fragile, conflict-affected and vulnerable settings, and that trust is an essential success factor for any AAP activities, these should be carried out in a conflict-sensitive way. This means that programmes are designed and implemented in a way that proactively seeks to mitigate the risks of inadvertently exacerbating social tensions, contributing to conflict, or undermining factors of social cohesion in each society or community. This is also referred to as the 'do no harm' principle.

Prevention and response to sexual exploitation, abuse and harassment (PRSEAH)

Acts of sexual exploitation and abuse (SEA) are the most egregious breaches of accountability by the humanitarian community. Staff are required to report SEA by humanitarian workers when it happens, and to actively prevent and respond to it. PRSEAH should be considered across the work we do

with affected people, including ensuring people are aware of their rights, the expected behaviour of humanitarian staff, and how affected people can safely and confidentially make a report if their rights have been violated. WHO uses the term PRSEAH in its 2023 strategy *Preventing and responding to sexual misconduct* (18), whereas other UN agencies use PSEA. The IASC diagram at Fig. 4 outlines the links between AAP and PSEA.

Fig. 4 AAP and PSEA linkages – results diagram



AAP: accountability to affected people; PSEA: protection from sexual exploitation and abuse.

Source: IASC (19)

2.1 Further reading

IASC operational guidance on data responsibility in humanitarian action. Geneva: Inter-Agency Standing Committee; 2023 (<https://interagencystandingcommittee.org/operational-response/iasc-operational-guidance-data-responsibility-humanitarian-action>, accessed 27 November 2024).

IASC guidelines, inclusion of persons with disabilities in humanitarian action. Geneva: Inter-Agency Standing Committee; 2019 (<https://interagencystandingcommittee.org/iasc-guidelines-on-inclusion-of-persons-with-disabilities-in-humanitarian-action-2019>, accessed 27 November 2024).

IASC policy on gender equality and the empowerment of women and girls in humanitarian action (updated). Geneva: Inter-Agency Standing Committee; 2023 (<https://interagencystandingcommittee.org/iasc-reference-group-gender-and-humanitarian-action/iasc-policy-gender-equality-and-empowerment-women-and-girls-humanitarian-action-updated>, accessed 27 November 2024).

WHO policy on preventing and addressing sexual misconduct. Geneva: World Health Organization; 2023 (<https://www.who.int/publications/m/item/WHO-DGO-PRS-2023.4>, accessed 27 November 2024).

Good practice note: conflict sensitivity, peacebuilding and sustaining peace. New York: United Nations Sustainable Development Group; 2022 (<https://unsdg.un.org/resources/good-practice-note-conflict-sensitivity-peacebuilding-and-sustaining-peace>, accessed 27 November 2024).

Chapter 3 AAP in WHO's emergency response framework

WHO's *Emergency response framework (ERF) edition 2.1 (5)* outlines that "WHO's primary accountability is to the populations it serves, including the principles enshrined in the Accountability to Affected Populations commitments."

In all the critical functions of the ERF, WHO has responsibility for ensuring that emergency responses are accountable to affected people. Table 1 to Table 7:

- summarize AAP responsibilities across the critical functions;
- indicate the chapters in this Handbook that provide more information to support the function in fulfilling these; and
- detail indicators to enable WHE teams at headquarters and regional offices to track the implementation of AAP commitments across emergency response countries through the reflection of AAP in standard planning, monitoring and reporting documents.

Annex 5 details indicators that can be used in assessing AAP from the community perspective, and these should be used by country offices to assess AAP impact in country.

3.1 Leadership

This function includes incident management, PRSEAH, communication and external relations.

Table 1 Leadership responsibilities

AAP Responsibilities	For more detail, refer to...	AAP indicators
• Hold ultimate responsibility for AAP in WHO emergency operations. • Oversee process to embed AAP into strategic and operational plans and budgets, and into monitoring and evaluation processes. • Oversee AAP activities incorporating PRSEAH requirements. • Assign AAP responsibilities across the incident management team and to an AAP focal point where required. • Ensure an AAP workplan is established, including targets, and regularly review progress. • Ensure community representatives are involved in key decision-making bodies such as the incident management system pillars and health cluster. • Encourage and support WHE staff to engage directly with affected communities. • Champion the voice of affected people, using data from participatory approaches and CFMs, in internal and external decision-making forums. • Advocate for inclusion of AAP approaches with governments. • Ensure WHE involvement in AAP coordination bodies and collective AAP approaches (e.g. collective/interagency CFM). • Ensure donor proposals, logframe indicators, budgets and reports include AAP activities and respond to donor AAP priorities. • Ensure external communication products reflect activities and outcomes of AAP activities.	Section 4.4 Programme design Section 4.6 Monitoring Section 4.8 Evaluation Section 8.1 Working with governments Section 8.3 Collective AAP Annex 1 Suggested AAP roles and responsibilities across the incident management team Annex 2 AAP focal point terms of reference Annex 3 AAP workplan template Annex 4 AAP in donor proposals Annex 5 Beneficiary-focused AAP indicators Annex 6 AAP in the operational plan Annex 7 Immediate AAP activities for a sudden onset emergency	• Country office operational plans include AAP activities, indicators and budget. • Country office emergency donor proposals include AAP activities, indicators and budget.

AAP: accountability to affected people; CFM: complaints and feedback mechanism; WHE: Health Emergencies Programme.

3.2 Partner coordination and engagement

This function includes health and intersectoral coordination and liaison.

Table 2 Partner coordination and engagement responsibilities

AAP Responsibilities	For more detail, refer to...	AAP indicators
- Ensure health cluster, sector or outbreak coordination group embeds AAP in all the key functions and all the steps of the humanitarian programme cycle according to Global Health Cluster <i>AAP – operational guidance (6)</i>. - Ensure community representatives are involved in the health cluster. - Empower national and local actors to lead elements of the response through targeted capacity-building initiatives, technical support and training programmes tailored to their needs.	Chapter 8 Partnerships, collective AAP and coordination Annex 1 Suggested AAP roles and responsibilities in the incident management team	- Health clusters/sectors have an AAP action plan or have integrated AAP into their overall strategy in a dedicated section. - Country office emergency operational plans detail localization actions to empower local actors (informed by upcoming WHO localization strategy).

AAP: accountability to affected people.

3.3 Health information and epidemiology

This function includes risk and needs assessment; surveillance and epidemiology; analytics; and information products.

Table 3 Health information and epidemiology responsibilities

AAP Responsibilities	For more detail, refer to...	AAP indicators
- Use participatory approaches and data sources in public health situation analysis and needs and risk assessments, including data from existing CFMs. - Ensure PHSAs and needs assessments seek to understand affected people's needs and priorities about receiving information, participating in decision-making and accessing CFMs, and there are existing mechanisms and strategies in place that can be used to achieve these. - Report on AAP activities, data and resulting programme adaptations in situation reports (sitreps) and annual country reports. - Ensure voices of affected people feed into all relevant aspects of information management and the public health information services implemented in the emergency, including needs assessment, emergency surveillance systems and mortality estimation.	Chapter 8 Partnerships, collective AAP and coordination Section 4.2 Situation analysis and needs/risk assessment Section 4.7 Reporting Chapter 5 Participation and inclusion Annex 8 AAP in PHSA Annex 9 AAP questions for needs assessment	- PHSAs reflect data sources from inclusive participatory processes, including CFMs, or identify this as a gap. - PHSAs include information on existing successful strategies and mechanisms in place to ensure participation and inclusion, information provision and transparency, and complaints and feedback, or identify this as a gap. - Country office AAP activities, data and programme adaptations are reported in sitreps and annual country office reports.

AAP: accountability to affected people; CFMs: complaints and feedback mechanisms; PHSA: public health situation analysis.

3.4 Planning and monitoring

This function includes action plans; strategic response plans; monitoring and evaluation; risk management; and emergency response reviews.

Table 4 Planning and monitoring responsibilities

AAP Responsibilities	For more detail, refer to...	AAP indicators
- Embed AAP actions in WHO planning processes and documents including emergency operational plans and strategic response plans. - Embed AAP in WHE and joint partner planning processes and documents, including joint partner operational plans. - Ensure planning, monitoring and evaluation processes use inclusive, qualitative, participatory approaches. - Embed a review of AAP in monitoring and evaluation processes and use data, analysis and lessons learned to adapt programmes. - Plan, implement and oversee CFM, including processing of sensitive complaints.	Section 4.4 Programme design Section 4.6 Monitoring Section 4.8 Evaluation Chapter 5 Participation and inclusion Chapter 7 Complaints and feedback Annex 2 AAP focal point terms of reference Annex 3 AAP workplan template Annex 5 Beneficiary-focused AAP indicators Annex 6 AAP in the operational plan Annex 10 AAP in evaluations and JORs	- Country office emergency operational plans describe consultations undertaken with affected people during the planning process. - Country office emergency operational plans outline how WHO will ensure a functioning CFM. - All joint operational reviews (JORs) led by headquarters/regional WHE teams include review of country office progress towards AAP commitments.

AAP: accountability to affected people; CFMs: complaints and feedback mechanism; JORs: joint operational reviews.

3.5 Technical expertise and health operations

This function includes prevention and control; risk communication and community engagement; health service delivery; technical expertise; and training of health staff.

Table 5 Technical expertise and health operations responsibilities

AAP Responsibilities	For more detail, refer to...	AAP indicators
- Use AAP data from assessments in programme design, and engage with affected people in design, targeting and implementation. - Plan and implement AAP messaging, including on PSEA, alongside programme implementation. - Adapt programmes in line with needs and priorities of affected people, identified through CFM and participatory approaches. - Train health staff on AAP and PSEA requirements.	Section 4.4 Programme design Section 4.5 Emergency response implementation Chapter 5 Participation and inclusion Chapter 6 Information provision and transparency	- Country office emergency operational plans outline mechanisms for ongoing participation of affected people in decision-making. - Country office emergency operational plans outline AAP and PSEA information provision plans.

AAP: accountability to affected people; CFMs: complaints and feedback mechanism; PSEA: protection from sexual exploitation and abuse.

3.6 Operations support and logistics

This function includes supply chain management, field support and health logistics.

Table 6 Operations support and logistics responsibilities

AAP Responsibilities	For more detail, refer to...	AAP indicators
- Content and quality of equipment, supplies, technical expertise and medical services for affected people are adapted to reflect feedback received. - Timing and method for provision of equipment, supplies, technical expertise and medical services for affected people are adapted to reflect feedback received.	Section 4.4 Programme design Section 4.5 Emergency response implementation Section 4.6 Monitoring Section 7.6 The complaints and feedback loop	JORs, sitreps and annual country reports outline how specifications and delivery of equipment, supplies, technical expertise and medical services have been adapted to feedback received.

AAP: accountability to affected people; JORs: joint operational reviews.

3.7 Finance and administration

This function includes grant management, procurement, and human resources and surge.

Table 7 Finance and administration responsibilities

AAP Responsibilities	For more detail, refer to...	AAP indicators
- Budget for AAP activities. - Support Incident Manager/WHE Coordinator to ensure AAP human resource capacity exists within the incident management team, or request standby partner capacity if required. - Ensure partner agreements, standard operating procedures (SOPs), contractual modalities and reporting consider and include AAP.	Section 8.2 NGO implementing partners	- AAP activities are represented in operational plan budgets. - AAP activities are reflected in implementing partner agreements and reports.

AAP: accountability to affected people.

Chapter 4

AAP in the humanitarian programme cycle

Affected people, in all their diversity, are involved in and inform all parts of the humanitarian programme cycle.

Why is this important?

The programme cycle is the operational framework that helps ensure humanitarian responses remain relevant to, and effective for, the people they serve. AAP principles must be an integral part of each stage of the programme cycle to ensure that WHO systems and processes plan, design, manage and measure health assistance programmes that respond to the diversity, expressed views and rights of affected communities.

Minimum actions

- Emergency preparedness activities consider the needs, priorities and rights of affected people to effectively prepare for a people-centred response.
- Situation and needs analysis are people-centred, ensure the views of affected people in all their diversity are included, and collect information needed to plan AAP activities.
- Programme design responds to affected people's expressed needs and priorities, affected people are involved in design decisions and AAP activities are integral to programme design.
- Programmes are implemented with the participation of affected people and are flexible enough to adapt to feedback.
- Monitoring includes the collection of qualitative, quantitative and CFM data from affected people and involves the participation of affected people.
- Evaluations use community feedback data on quality and impact of the programme and engage affected people in the evaluation process.
- Internal systems exist for management to be informed of and take decisions that respond to feedback.
- Internal and external reporting includes data from, and actions taken because of, participation and feedback activities.

4.1 Disaster and crisis preparedness

Emergency preparedness planning allows WHO to be able to respond faster and more effectively when there is an emergency. AAP should be integrated into preparedness planning in the following ways:

- Triangulate existing data to inform pre-crisis analysis in the following areas:
 - Understand vulnerabilities and capacities of different groups and who are the most at risk.
 - Understand peace and conflict dynamics and ensure preparedness planning considers, and does not unintentionally exacerbate, community tensions.
 - Understand the existing communication channels used by the community as well as their information needs, and anticipate how these could be affected by different emergency scenarios.
 - Understand participatory structures and processes that can be activated in an emergency.
 - Map existing CFMs (i.e. collective/interagency mechanisms) that could be scaled up in an emergency, or where they do not exist, plan the establishment of a feedback system.
- Define how access to health services will be secured for the most vulnerable groups, such as people with disabilities, elderly, children, pregnant women, people in remote locations or any other vulnerable groups in the country context.
- Embed activities and approaches across WHO's AAP commitments in preparedness plans and budgets.
- Align AAP actions with ministry of health and health cluster preparedness plans.
- Establish focal persons and clear roles and responsibilities for AAP that can be activated in an emergency (see Annex 1 and Annex 2).
- Train staff and partners in AAP as a key part of preparedness activities.
- Ensure staff, partners and the community know about preparedness plans.

4.2 Situation analysis and needs/risk assessment

Having a good understanding of the context in which people live is the basis for designing effective programmes. It helps us anticipate the evolution of people's needs as a crisis hits and unfolds.

WHO and the Global Health Cluster have several public health information approaches and tools (20) that feed into understanding and assessing the health situation and needs of a given population during an emergency. These tools and approaches must ensure that the views of affected people are reflected, and that key information needed to plan AAP activities is collected and reflected.

4.2.1 PHSA

The PHSA (21) is a key document that highlights emergency- and crisis-related health risks. Based on secondary data, it characterizes and quantifies epidemiologic conditions, existing health needs and possible health threats faced by the crisis-affected population, including vulnerable and marginalized groups. It identifies the major areas for health action to respond to and recover from the crisis at hand and identifies major population groups at risk.

AAP should be reflected in the PHSA in the following ways:

- As far as possible, use secondary data that reflects participatory approaches and the expressed views of affected people, including the most marginalized.
- Highlight population-related risks or specificities.
- Identify pre-existing strategies used to involve communities in health-related decision-making.
- Identify pre-existing public health communication strategies that could be used to communicate AAP and PSEA messages to affected people (see Chapter 6).

- Reflect information on pre-existing CFMs that could be used to enable affected people to reach out to WHO with questions, feedback or complaints.

Annex 8 outlines in more detail the AAP information to be included in different sections of the PHSA document.

PHSA information that is relevant to the communities should be verified directly with affected people to identify gaps or inaccuracies to be updated in the needs assessment. This should be done using accessible and inclusive methods as outlined in Chapter 5.

4.2.2 Health or multisector needs assessment

Needs assessments fill gaps in or update existing data on the specific needs of affected people, as well as provide more specific information to support programme design. The following should be considered when planning and implementing needs assessments from an AAP perspective:

- Coordinate assessments with partners and other sectors to avoid assessment fatigue in communities, and ensure effective and collaborative use of existing data.
- Engage affected people and local partners in designing participatory needs assessments that reach the most at-risk people, and that address identified gaps and inaccuracies from the PHSA. Communities should also be involved in data interpretation and validation.
- During the needs assessment, inform affected people why you are collecting information from them, how it will be used and how the results of the needs assessment will be made available to them. Do not raise expectations about the level of assistance that will follow the needs assessment.
- Ensure particularly marginalized groups are adequately considered in needs assessment sampling methodology, so that the information collected is representative.
- Ensure data collected can be disaggregated by location, gender, age and disability, at a minimum. This disaggregation is important in designing adequate programme adaptations for different groups.
- Design questionnaires to incorporate perceived needs questions, such as in the Humanitarian Emergency Settings Perceived Needs Scale (HESPER) (22).
- Include qualitative data collection methodologies to allow people to express their needs from their own perspectives.

Needs assessments should also assess people's information and communication needs, preferences and barriers to inform the design and delivery of AAP strategies. Key questions to include in needs assessments from an AAP perspective are:

- What are people's preferred communication channels?
- Do people know how to provide feedback or lodge a complaint about programmes or the humanitarian response, and how would they prefer to do this?
- Are people, in all their diversity, receiving all the information they need to access humanitarian assistance?
- Are people satisfied with their level of engagement in programme decision-making? What are their preferences in terms of participation?
- Are the people most in need receiving assistance and accessing services?
- Are people satisfied with the timeliness, relevance, appropriateness and quality of humanitarian assistance?
- Are humanitarian actors treating affected people with respect and dignity?

Annex 9 details more example AAP questions to be included in needs assessments.

4.3 Further reading

Menu of accountability to affected populations (AAP) related questions for multi-sector needs assessments (MSNAs). Geneva: Inter-Agency Standing Committee and REACH; 2018 (https://interagencystandingcommittee.org/sites/default/files/migrated/2018-06/reach_iasc_aap_psea_task_team_menu_of_aap_questions_for_needs_assessments_june_2018.pdf, accessed 27 November 2024).

Community engagement and accountability toolkit: tool 13, CEA in assessments tool. Geneva: International Federation of Red Cross and Red Crescent Societies; 2022 (<https://www.ifrc.org/document/cea-toolkit>, accessed 27 November 2024).

Pocket guide: information and communication questions in rapid needs assessments. London: CDAC Network and ACAPS; 2014 (<https://static1.squarespace.com/static/60996b757eb6521a42f3839d/t/61bd582d71f7db616098ed62/1639798830847Pocket+Guide+Information+%26+Communication+Questions+in+Rapid+Needs+Assessments+%28coloured+version%29.pdf>, accessed 27 November 2024).

The Washington Group: short set on functioning (WG-SS). Washington: Washington Group on Disability Statistics; 2022 (<https://www.washingtongroup-disability.com/question-sets/wg-short-set-on-functioning-wg-ss/>, accessed 27 November 2024).

4.4 Programme design – strategic and operational planning

Programme design should address the needs and priorities identified by affected people during needs assessments or when registering their opinions through CFMs. Translating their expressed needs into concrete action builds and maintains trust with affected communities.

- Involve affected people in the design process, so that programmes are co-created or a design is checked with them, so it can be adjusted if necessary, especially by and for marginalized groups. This will require a flexible approach and being willing to adapt the usual way of doing things.
- Ensure the allocation of adequate resources for implementing AAP actions.
- Build flexibility into the programme design, as well as into funding proposals and grants contracts, in order to respond to monitoring data or feedback from affected people during implementation of the programme.

- Affected people should feed design decisions into the programme, such as:
 - identifying the geographical areas most in need;
 - identifying target groups and designing selection criteria;
 - determining the contents and specification of packages/items for distribution;
 - deciding on the most appropriate timing, frequency and location for activities;
 - prioritizing the most urgent intervention within a limited budget;
 - determining the most suitable adaptations to activities to ensure access of different groups;
 - designing relevant information messages and identifying the best communication channels;
 - planning how you will ensure the regular participation of affected people in decision-making; and
 - developing or improving accessible CFMs.

Programme planning documents, such as the WHO emergency operational plan, should describe how meaningful consultations have informed design and outline how WHO plans to implement AAP principles and commitments. See Annex 6 for more detail.

The humanitarian response plan should include similar information to the above and reflect health cluster plans.

4.5 Emergency response implementation

Designed activities are delivered, either directly by WHO or by implementing partners.

- AAP systems and activities related to participation and inclusion, information provision and transparency, and complaints and feedback are implemented. Affected people are actively informed of their existence. See chapters 5, 6 and 7 for the specifics of implementing these three areas.
- The type of activities implemented will depend on where the country office is on their AAP journey: a) taking the first steps, b) putting in place the AAP systems, processes and activities, or c) learning, improving and adapting their AAP approach. Annex 11 outlines what activities at these different stages look like.
- AAP activities should be embedded in emergency operation workplans with responsibilities assigned across teams and progress tracked by emergency response leadership.
- A specific AAP workplan should be drafted by the AAP focal point. This will break activities down into more detail, assign responsibilities across the critical functions and specific teams, and enable a closer tracking of progress during implementation. The workplan should touch on all aspects of WHO AAP commitments and responsibilities across ERF functions. See Annex 3 for an AAP workplan template.
- Programme implementation should be done, where possible, with the meaningful participation of affected people, including vulnerable and marginalized groups. In the case of medical programme delivery, these could be peripheral activities. For example, village health committees have responsibilities for the provision of health information, including awareness raising for pregnant women on the importance of antenatal care. Community health committees feed into surveillance activities.

4.6 Monitoring

AAP should be embedded in monitoring processes and tools to track the quality and accountability of programmes over time and to identify areas for improvement and course correction. This should include process monitoring of beneficiary experiences of humanitarian health programmes as well as beneficiary outcome monitoring, whether implemented by WHO or partners.

4.6.1 Choose indicators to reflect your AAP objectives and plans

- Include AAP indicators in your operational plan, partnership agreements and donor logframes. AAP indicators should cut across all programme areas. Annex 5 includes some suggestions for AAP indicators that you can use.
- Affected people should be involved in designing indicators or choosing the indicators that they feel would best describe a successful health intervention from their perspective.

4.6.2 Collect data to measure progress

Data should be collected directly from affected people to produce an accurate understanding of their experiences.

- Involve affected people in designing and implementing monitoring activities.
- Include questions to monitor AAP in programme monitoring tools used to collect data from affected people.
- Design new data collection tools, such as patient satisfaction surveys, to better understand affected people's experiences of health assistance.
- Include qualitative data collection methods, such as focus group discussions, to allow for the exploration of different opinions as well as potential improvements. This should include WHO staff going to speak to affected people in the communities they support.
- Remember to use data that is already generated through programmes. For example, analysing data on the distribution of assistive equipment to people with disabilities could provide insights into who has been reached and who has not. Another key example is CFM data, which could show levels of satisfaction or dissatisfaction with certain programme components.
- Include in JORs a review of AAP systems and processes used in country office emergency responses. See Annex 10 for questions that can be included in JORs from an AAP perspective.

4.6.3 Choose what data to collect

Ask questions about:

- knowledge and effectiveness of AAP activities including information provision, CFMs and participatory processes;
- people's experiences of WHO programme activities and any barriers they experience in accessing them; and
- suggestions for improvements in how programmes, or programme components, are delivered.
- Analyse and reflect on data collected
- Establish practices for regular analysis and review, together with relevant teams and management, to understand progress made and to make programmatic decisions based on the data.
- Involve affected people in reflecting on and validating monitoring data and feeding into programme adaptations.
- Disaggregate data by location, sex, age and disability, at a minimum, to get an understanding of the experiences of different vulnerable groups across locations, and to monitor the effectiveness of any adaptations made for particular groups.
- Regularly analyse complaints and feedback data and feed it into programme decision-making and course correction.

4.7 Reporting

Achievements and challenges with implementing AAP activities should be captured and reported.

4.7.1 Reporting to the community

Where changes to a programme were made based on feedback collected from affected people, including complaints and feedback data, this should be documented and shared with affected people. Where changes were not made, for example because of contextual challenges, these should also be shared. This information could be integrated into community-facing FAQs used by field staff, partners and CFMs. See Chapter 6 for more information on communication channels.

4.7.2 WHO reporting

It is important WHO captures and communicates AAP achievements among country offices to support sharing of good practice, to track global progress towards IASC and Grand Bargain commitments, and to share achievements with Member States and donors. The following should be included in reports:

- how the needs and concerns of affected people, in all their diversity, have been heard, for example, focus group discussions held, satisfaction surveys conducted, or CFMs operating;
- how many people were involved and some demographic breakdown to show that these activities were inclusive;
- what affected people expressed through the above opportunities, what their major questions, concerns or suggestions were, and how these differed by location or vulnerable group;
- how these questions, concerns or suggestions have been responded to, at an individual level and response/programme level; and
- what adaptations were made in response to the feedback from affected people.

This information should be reflected in the reports being prepared and disseminated by country offices, including sitreps, annual country office reports and donor reports.

4.8 Evaluation

Evaluation is a key tool in AAP, as it assesses the extent to which resources at WHO's disposal were used effectively to meet the needs of an affected population. Evaluations provide an opportunity to reflect, learn, adapt and improve. The voices and perspectives of affected people are crucial in evaluating the outcomes of a humanitarian programme and developing lessons learned, as outlined in the 2023 WHO *Practical guide to evaluation for programme managers and evaluation staff* (23).

See Annex 10 for questions that can be included in evaluations from an AAP perspective, mapped to the evaluation criteria outlined in the WHO *Evaluation policy 2018* (24).

4.9 Further reading

Community engagement and accountability toolkit: tool 7, CEA monitoring and evaluation tool. Geneva: International Federation of Red Cross and Red Crescent Societies; 2022 (<https://www.ifrc.org/document/cea-toolkit>, accessed 27 November 2024).

AAP toolkit: tool 5.8, AAP indicators and monitoring guidance. Geneva: UNICEF; 2020 (<https://www.corecommitments.unicef.org/kp/tool-5.8-aap-indicators-and-monitoring-guidance.pdf>, accessed 27 November 2024).

Practical guide to evaluation for programme managers and evaluation staff. Geneva: World Health Organization; 2023 (<https://iris.who.int/handle/10665/373678>).

DG ECHO protection mainstreaming key outcome indicator and monitoring tool. Brussels: European Commission; 2021 (<https://www.dgecho-partners-helpdesk.eu/download/referencedocumentfile/204>, accessed 27 November 2024).

Accountability to affected populations [website]. US Department of State, Bureau of Population, Refugees, and Migration (<https://www.state.gov/other-policy-issues/accountability-to-affected-populations/>)

Chapter 5

Participation and inclusion

Affected people, in all their diversity, can meaningfully participate in decisions that affect the provision of emergency health assistance.

Why is this important?

Participation enables affected people to take a lead role in identifying and addressing their health priorities, promoting a dignified approach that puts them at the centre of humanitarian response. In order for the differing and intersecting needs of people to be taken into account, this participation should be inclusive of affected people in all their diversity.

Minimum actions

- Affected people are made aware of their right to participate in decisions on humanitarian health provision and how they can do this.
- All affected people can participate equally in identifying and addressing their health priorities.
- Means of continuous participation across the programme cycle are adapted to ensure accessibility for marginalized people, or people with specific needs.
- Programme staff regularly visit affected communities to understand changing needs and concerns.
- Existing community consultative platforms are used and strengthened, or established where these do not exist.
- Consultations with affected people are documented, reported into decision-making spaces and used to inform programme decisions.

5.1 Understanding existing community structures and power relations

It is important to understand the composition of communities, conflict dynamics, existing structures for decision-making, and which groups tend to have fewer opportunities to contribute to community decision-making. This information can be found from secondary sources as part of a situation analysis. Knowing this can help in understanding which voices will be the strongest and which marginalized groups WHO needs to seek out to make sure their needs also feed into programmatic decisions – and to ensure a conflict-sensitive approach.

5.2 Levels of participation

There are different levels of participation that affected people can have in programmes. The below (25) can help in understanding where current programmes are on this scale, and what to aim for.

- **Inform (e.g. noticeboards):** WHO plans and implements programmes according to their understanding of needs, informs affected people about plans, but does not provide any opportunity to contribute to these plans.
- **Consult (e.g. assessment):** WHO asks affected people for their perspectives through surveys or discussions and takes this information into account in decision-making, but affected people do not have any power to influence the decisions WHO takes.
- **Involve (e.g. community meetings):** affected people work with WHO to have an input into decisions and provide feedback, but WHO develops the plans and makes the final decisions.
- **Collaborate (e.g. community committee):** affected people and WHO enter a partnership, sharing responsibility for decision-making. Where WHO is not able to implement a decision requested by affected people, it provides an explanation as to why this is, but WHO still makes the final decisions.
- **Empower (e.g. community action plan):** affected people have complete control over identifying problems and developing solutions, as well as guiding the implementation of these.

Within health programmes, certain elements require highly technical personnel with medical, scientific or other qualifications, so it may not be possible to hand over complete control to communities for entire programmes. However, there may be elements of programmes where communities can take complete control, and these should be assessed and considered.

As outlined in Chapter 4, **it is important to ensure that affected people participate in decisions across the programme cycle**. WHO should aim at a minimum to ‘consult’ with affected people in these different processes, but ideally should aim to ‘collaborate’ with them.

5.3 Structures and opportunities for participation

It is important that participation does not become a burden on affected people and, where possible, uses existing community structures, such as:

- **Village health committees:** where participatory health structures already exist close to communities, work with these to feed into different programme decisions. Be aware of how representative these committees are of more marginalized groups and plan for the input of those that are not represented.
- **Groups for marginalized or vulnerable persons, such as women, youth or people with disabilities:** there may be informal structures that can provide advice on accessing more marginalized or hidden groups of people. They may have regular meetings that you could ask to participate in or attach a consultation session to.
- **Existing participatory structures of health partners:** other humanitarian health partners may have created participatory structures, such as community committees, to support them with ensuring affected people’s involvement in decision-making. Rather than establishing something new, WHO could discuss how these structures could be used as a consultative platform for all health cluster partners.

If the above structures are not available or additional opportunities are needed, then you can also consider:

- **Establishing community committees** that are representative of the people WHO is trying to reach. These could have responsibility for implementing parts of the programme, for disseminating information, or for monitoring programme progress from the community perspective and identifying gaps and challenges.
- **Regular community meetings** where many people can attend. This is an opportunity to share information, ask and answer questions, and receive feedback and input.
- **Key informant interviews** with representatives of different populations groups or civil society organizations to gather different perspectives.
- **Round table meetings** with representatives of different population groups to prioritize and agree ways forward.
- **Adapting assessment tools/techniques** to make sure that the most vulnerable and marginalized feel safe and comfortable to contribute their opinions.
- **Focus group discussions** where different groups of people can have less formal conversations and express their specific health concerns and challenges.
- **Working with local civil society organizations** which are based within the communities where WHO works, and who have a good understanding of specific marginalized groups.
- **WHO staff regularly visiting communities** and engaging in informal communication with people to gain an up-to-date understanding of challenges and needs within the community.

5.4 Overcoming organizational challenges to participation

Table 8 suggests solutions for overcoming organizational challenges that can impede participation.

Table 8 Participation: challenges and solutions

Challenge	Solutions
Lack of time	• Undertake a rapid consultation process in one day. • Identify ongoing activities, monitoring exercises or meetings that could be utilized to ask a few additional questions. • Speak by phone to community leaders or community-based organizations that represent affected people. • Use remote digital methods such as SMS or social media. • Use secondary consultative data from other health partners or partners from other sectors. • Reflect on relevant data from existing CFMs. • Build flexibility into the programme design, so it can be adjusted when there is time to undertake consultations with communities. • Build time for participation in implementation, monitoring and evaluation into the design.

Lack of access to affected people	Use remote digital methods such as SMS or social media. Speak by phone to community leaders or community-based organizations that represent affected people. Work with volunteers or outreach staff from local organizations who can access affected people.
Lack of funds	Plan resources for participation in advance and allocate in core budget. Attach small consultations to existing programme activities, or near programme sites, to keep costs down. Use remote digital methods such as SMS or social media. Speak by phone to community leaders or community-based organizations that represent affected people.
Lack of donor flexibility on how funds can be used	Use information from consultations to advocate with donors for more flexibility to meet affected people's expressed needs. Be honest with communities about donor restrictions, and focus participatory decision-making on the areas of flexibility.
Affected people's opinion clashes with WHO technical opinion	Be prepared to listen to the affected people's concerns and be flexible with your technical opinion and approach. Have a conversation with affected people in which you outline the challenges with their suggested approach and ask if they have alternative suggestions that address these challenges. Provide communities with alternative approaches and ask for their preferred option. Be prepared to discuss any aspects they are concerned about.
Affected people's voices conflict with government voices	Understand what the government restrictions or red lines are and why, and where there could be flexibility. Be honest with communities about these restrictions and focus participatory decision-making on the areas of flexibility. Understand the details of community concerns with the proposed government approach. This could provide insights into potential complementary programmes needed by other health partners, adaptations needed for different groups or in different locations, or the need for clearer information to be provided.
Affected people's voices conflict with each other	Understand the specifics of the issues the conflicting voices are centred around, and why people have conflicting opinions. If the differences centre around different groups of people (e.g. women have different opinions to men) or locations (e.g. rural communities have different opinions to urban communities), then the programme may need to adapt its approach for different groups or areas. If programme areas under discussion cannot be adapted or need to be prioritized, then be honest about that and seek community support in identifying a fair way to prioritize.

CFMs: complaints and feedback mechanisms; SMS: short messaging service.

5.5 Supporting marginalized groups to participate

Table 9 lists ways of supporting various marginalized groups to engage in participatory processes.

Table 9 Ways to support participation among marginalized groups

Population group	Ways to support participation
People living with serious or long-term medical conditions	Engage people at health facilities if they are attending regularly or staying long term, and ask the best way to engage with people there. Arrange spaces for participation in places where people can physically access, feel safe and comfortable, and attend at times that are convenient for them, or conduct activity in people's homes. Ensure groups are small to encourage people to participate. Plan for short sessions so as not to tire people out or cause discomfort. Support carers to attend when needed. Make sure people are aware of their right to participate and voice their opinions.
Women	Consult with communities on the most appropriate way to engage with women, including what their main barriers to participation are and how to overcome them. Collaborate with organizations that specialize in working with women, such as local NGOs, or UN Women and UNFPA. Establish separate spaces for women to participate, led by female staff. Arrange spaces for participation in places where women can physically access, feel safe and attend at times that are convenient for them. Make sure women can bring any young children they are caring for and the space can safely accommodate them. Work with existing women's groups and spaces. Make sure the women are aware of their right to participate. Encourage men and religious leaders to support women's participation. Use practical examples related to the delivery of more effective health care. Avoid 'rights-based' language if it would create more barriers.
Children	Collaborate with organizations that specialize in working with children, such as UNICEF. Ask teachers and parents for advice on the best way for children to participate. Use child-friendly approaches in spaces where children will feel safe expressing themselves. Split children into sub age groups and use appropriate methods with each.

Population group	Ways to support participation
People with disabilities	Collaborate with disabled people's organizations to help plan and support their participation and provide access to people at community level. Ensure physical spaces for participation are accessible including travel to the specific location, or move the participation to a place where people already travel to, or conduct the activity in people's homes. Support carers to attend when needed. Ensure communication channels are adapted for different impairments so that everyone can participate. Make sure they are aware of their right to participate.
Elderly people	Collaborate with older people's organizations to help plan and support their participation and to access people at community level. Make sure spaces are accessible and comfortable for older people, or conduct the activity in people's homes. Support carers to attend when needed. Ensure groups are small to encourage people to participate. Make sure they are aware of their right to participate.
LGBTIQ+ people	Collaborate with LGBTIQ+ organizations to help plan and support their participation and access to people at community level. Identify private and safe spaces, or conduct activity in people's homes. Focus on data privacy and protection, i.e. collect minimal personal information (preferably none), including photographs, especially in countries where their preferences are criminalized or put them at high risk. Create safe and stigma-free spaces, selecting facilitators carefully. Use the opportunity to provide people with information about general and specific health services that are available to them.

LGBTIQ+: lesbian, gay, bisexual, transgender, intersex or queer; NGO: nongovernmental organization; UN: United Nations; UNFPA: United Nations Population Fund; UNICEF: United Nations Children's Fund.

5.6 Further reading

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Community engagement and accountability toolkit: tool 17, community meetings tool. Geneva: International Federation of Red Cross and Red Crescent Societies; 2022 (<https://www.ifrc.org/document/cea-toolkit>, accessed 27 November 2024).

Participatory techniques flipbook: different ways to have conversations with different people. Geneva: International Committee of Red Cross; 2019 (<https://www.icrc.org/en/document/different-ways-have-different-conversations-different-people>, accessed 27 November 2024).

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Chapter 6

Information provision and transparency

Ensure transparent communication with affected people on key programme information and respond to their priority information needs, using channels that are equitable, appropriate and accessible.

Why is this important?

Receiving accurate information about humanitarian organizations, the assistance they provide and affected people's rights enables affected people to make informed decisions for themselves and their families, hold humanitarian actors to account and manage expectations about the assistance that can be provided. Providing accurate and up-to-date information can combat confusion, mistrust, rumours and misinformation.

Minimum actions

- Regularly consult with affected people, in all their diversity, to identify their information needs, preferred channels and language, and use this information to design information provision activities.
- Share information to affected people on health issues, programme objectives and activities, selection criteria, timing and location of services, and their rights and entitlements, including their right to provide feedback and make a complaint.
- Share information on available feedback and complaints channels, their scope and how to use them.
- Inform affected people of the expected standards of conduct by WHO and partner staff, including PSEA messaging, and how to report inappropriate actions or behaviours.
- Adapt information to make it accessible for different groups, including children and people with specific needs.

6.1 Understanding the communication context

Gain an understanding of what communication systems exist and are readily available to the communities WHO is trying to reach. For example, find out if there is reliable mobile infrastructure and what percentage of the population have access to mobile phones. Understand if there are common ways for sharing and receiving information, such as radio. An AAP working group in country may have collected this information, or know where to find it. Alternatively, CDAC Network releases Media Landscape Guides for different response countries, which provide significant insights into the communication context (see section 6.6).

6.2 Understanding the communication preferences and information needs of affected people

Needs assessment should include questions to give you a better idea of people's preferences and needs. These questions can be included in WHO needs assessments, or health cluster or multisector needs assessments. Annex 9 provides example needs assessment questions related to information and communication preferences.

Make sure to collect this needs assessment information from diverse groups in diverse locations to get an understanding of differing needs and preferences.

6.3 What information should WHO share?

- **Information about the organization:** What is the organization and its partners? How can the organization be identified? How can I contact them? What can I expect from the organization and its staff?
- **Information about the humanitarian assistance being provided:** What assistance is being delivered and how can it be used? Where and when is it being delivered? Who is eligible to receive assistance and who is not, and why? How do I get in touch if I need more information? Other information requested by affected people.
- **Information about affected people's rights:** Right to receive assistance free from any form of payment. Right to information about assistance. Right to provide feedback or raise a complaint about the assistance. Right to be treated with dignity and respect by aid workers.

Box 1

Information related to the integrity of WHO: protection from sexual exploitation and abuse, fraud and corruption

Making sure that affected people understand the behaviour they should expect from aid workers, their right to be treated with dignity and respect, and – if they are not treated in this way – how to make a confidential complaint are fundamental ways in which AAP activities support PSEA as well as WHO's efforts to prevent and respond to fraud and corruption.

Country offices should contact PSEA working groups in country for context-specific messaging to be used, available materials and advice on how to embed these into different programmes.

6.3.1 Frequently asked questions (FAQ) and answers

A community-focused FAQ document can form the basis of key messaging for affected people. Whenever programmes and services are established, change or are coming to an end, the FAQ document should be updated to reflect anticipated and actual questions from affected people.

Staff and partner staff with community-facing roles should be trained on FAQ content. It should also be used as part of a CFM to answer questions. It will ensure that information provided to affected people about WHO programmes is consistent and accurate.

Box 2

Health Resources and Services Availability Monitoring System (HeRAMS), AAP, and information provision and transparency

HeRAMS supports countries with the continuous analysis and dissemination of information on the availability of, and accessibility to, essential health resources and services in emergency response and fragile states.

Countries in which HeRAMS is operational should use the information it generates to provide affected people with up-to-date information on health service and resource availability. This could feed into a collective or WHO health call centre to provide people with up-to-date health referral information, or be communicated regularly through some of the other channels mentioned in this chapter.

6.4 Effective messaging

Effective messaging:

- has a clear goal;
- is short and simple, and does not contain more information than is needed;
- is written in the preferred language of the affected community and in multiple languages if needed;
- uses pictures to convey core messages where possible;
- uses terminology that has been tested with affected people for comprehension, avoids complex, technical language and uses local colloquialisms where appropriate;
- is coordinated and planned with other health and humanitarian actors to ensure consistency in terminology and content across the response; and
- is delivered respectfully to affected people.

6.5 Communication channels

When choosing which channels to use, consider the following:

- Use channels that are preferred by affected communities.
- Use channels that are accessible to marginalized groups, taking into consideration literacy levels, visual/auditory impairments, access to technology (e.g. phones and internet), and social/cultural barriers.
- Use a mix of channels, as it is unlikely that one channel will reach everyone, and always provide some opportunities for face-to-face communication.
- Use face-to-face or phone communication where the message is likely to trigger further questions or discussion on individual circumstances.
- Consider the most appropriate channel for the information to be shared, noting how the choice of channel may impact people's trust in the information. For example, is the information considered sensitive, or is the message for each individual or family different?
- Take advantage of times when there is already face-to-face contact with affected people as part of a programme activity to provide additional information.

Some communication channels that can be used are:

- SMS text message
- social media
- help line/call centre
- household visits by staff
- help desk at programme site
- verbally during medical appointments
- printed materials such as posters or leaflets
- radio programmes/information spots
- TV programmes/information spots
- TV screens or written materials in waiting areas
- community outreach or community meetings.

6.6 Further reading

Media and telecommunication landscape guides. In: CDAC Network [website]. London: CDAC Network (<https://www.cdacnetwork.org/media-landscape-guides>).

Community engagement and accountability toolkit: tool 14, Q&A sheet for volunteers. Geneva: International Federation of Red Cross and Red Crescent Societies; 2022 (<https://www.ifrc.org/document/cea-toolkit>, accessed 27 November 2024).

Community engagement and accountability toolkit: tool 19, communications methods matrix. Geneva: International Federation of Red Cross and Red Crescent Societies; 2022 (<https://www.ifrc.org/document/cea-toolkit>, accessed 27 November 2024).

Communicating with communities during the first 6 weeks of an emergency response. London: CDAC Network; 2014 (<https://www.cdacnetwork.org/tools-guidance/communication-with-communities-during-the-first-six-weeks-of-an-emergency>, accessed 27 November 2024).

WHO policy brief: building trust through risk communication and community engagement. Geneva: World Health Organization; 2022 (<https://iris.who.int/handle/10665/362670>).

Chapter 7

Complaints and feedback

Systematically receive and respond to complaints and feedback from affected people, in all their diversity, and adapt programmes according to feedback.

Why is it important?

Complaints and feedback mechanisms (CFMs) allow humanitarian actors to hear directly from affected people to understand their experiences of WHO programmes and enable us to support them in navigating and solving their challenges. They provide us with real-time data which can alert us to changes in the context, lack of knowledge about programmes, misinformation, unexpected oversights in implementation and many more issues.

Minimum actions

- Needs assessments identify accessible and preferred complaint and feedback collection mechanisms by consulting with affected people.
- WHO programmes and targeted communities are covered by a safe, accessible, inclusive CFM that ensures confidentiality and data protection.
- WHO is part of collective CFMs in contexts where these exist, function effectively, and meet the needs of affected people that WHO is supporting.
- Documented processes are in place for complaints handling, referral and response, including for sensitive complaints.
- Questions, complaints and feedback (including sensitive complaints) from affected people are addressed and closed in a timely manner, according to agreed SOPs.
- Complaints and feedback data is systematically analysed, reported into decision-making spaces, and used to inform programme design and course correction.
- Effectiveness, appropriateness, accessibility of and satisfaction with complaints and feedback processes are regularly monitored and used to improve the CFM.
- Actions taken in response to complaints and feedback from affected communities are reported back to them.

7.1 What is a CFM?

A CFM is a system that is dedicated to receiving, listening to and responding to the questions, views, feedback and complaints of affected people on what humanitarian organizations are doing or not doing. These could relate to:

- accessing health services or supplies
- levels of satisfaction with their experience of health care providers
- the relevance, quality and fairness of health care provision and supplies
- the behaviour of staff towards affected people
- information or misinformation on health issues or services.

7.2 Understanding complaints and feedback preferences

People affected by crisis and conflict may not feel they have the right to voice their opinion or complain. They may not feel safe to do so or may think it will have a negative effect on their ability to access assistance. It is important to consult with affected people on their preferred way of providing feedback or making a complaint, including sensitive complaints.

Annex 9 provides questions to ask to help assess these preferences. Make sure to consult with different segments of the population, who may have different preferences and access barriers.

7.3 Types of CFM

Complaints and feedback from affected people can be collected using different types of tools. Table 10 lists the most common tools and some pros and cons of each. It is worth noting that these same tools can also be used for providing information.

Table 10 Complaints and feedback collection tools

Type of CFM	Pros	Cons
Hotlines or call centres	• Can cover a large geographical area and population • Useful when there is limited regular access to affected people • Does not require people to travel or be in a particular location • Can be accessed by people with limited or no literacy • Call centre operators can have access to centralized databases of information to support them in responding to issues during a call	• Can cost affected people if it is not a toll-free number • Requires ownership of or access to a phone and credit to keep the phone active • The most vulnerable may not have phone access • Can be resource intensive to establish and run
SMS/WhatsApp	• Can cover a large geographical area and population • Useful when there is limited regular access to affected people • Does not require people to travel or be in a particular location	• Can only send short messages so may be difficult to explain or respond to issues in detail • Literacy is required • Those most at risk may not have phone access

Type of CFM	Pros	Cons
Information and feedback desks	• Allows for face-to-face communication and more effective resolution/referral of issues • More appropriate for receiving sensitive complaints • Can be positioned at programme sites so they are accessible • No literacy required	• Only accessible to those who can travel to the desk location • Can only operate at times when they are staffed
Outreach or monitoring staff (with data collection tablet)	• Face-to-face communication allows for better resolution of issues • More appropriate for receiving sensitive complaints • Community-facing staff are always prepared to collect feedback • Gives responsibility to more staff and helps them to better understand affected people's concerns	• Collecting and responding to feedback could distract staff from their main role
Social media	• Can cover a large geographical area and population • Useful when there is limited regular access to affected people • Does not require affected people to travel or be in a particular location • Affected people can learn from the queries of others • Good way to track and manage misinformation	• Tends to be used more by younger segments of the population in urban areas • Requires literacy • Not appropriate for sensitive complaints and protecting privacy • May need dedicated staff to process, respond to and analyse issues
Community meetings	• Can reach a large number of people at the same time • No literacy required • Affected people can learn from the queries of others • An opportunity for dialogue	• Not appropriate for sensitive complaints • Not useful for addressing individual concerns • Some people may dominate discussion and others feel less comfortable • Only accessible to those who can travel to the meeting location

CFM: complaints and feedback mechanism.

7.4 Find and connect with existing CFMs

Where possible WHO should join an existing CFM at country/response level rather than establishing something new. This is a more efficient use of resources, is less confusing for affected people, and contributes to WHO's IASC commitments to implement collective AAP solutions.

Understand what existing CFMs are already functioning in the context, for example, mechanisms within the ministry of health, CFMs run by other health partners, or intersectoral mechanisms at response level.

The AAP working group or humanitarian country team should be able to provide information on existing and planned collective CFMs. Alternatively, check directly with other UN agencies who may be running a CFM that WHO can join.

WHO should, at a minimum, be able to accept, process and close relevant health referrals from existing CFMs.

7.5 Requirements when joining or establishing a CFM

The following should be part of country office planning for any CFM that WHO establishes or joins.

- **Coverage:** the CFM should cover WHO's programmatic areas and their associated geographical areas, population groups and technical requirements.
- **Community awareness:** affected people need to know how they can contact humanitarian health organizations. They need to know the CFM exists, how to access it and their right to make a complaint. This should be part of WHO's information provision activities.
- **Accessibility:** there should be a choice of ways for affected people to access the CFM in order to cater for as wide a range of people as possible. Consider accessibility for the most marginalized groups such as elderly, people with disabilities, women and girls, LGBTIQ+ people, and people with serious medical conditions.
- **Staffing:** the required staffing should be assigned to design, implement and oversee the process of collecting and managing feedback and complaints. Programme staff and leadership who may be involved in receiving and responding to referrals from the CFM should also be sensitized on their role. Staffing for direct beneficiary communication should be gender sensitive.
- **Budget:** WHO should have the financial resources needed to establish or join a CFM.
- **Access to information and data:** staff receiving questions, issues or complaints should have access to information and data to respond to as many issues as possible on the spot. This should be through an FAQ document for more general questions. In the case of a call centre or information desk, use a connection to access databases to use when responding to queries about individual or family circumstances.
- **CFM database:** a centralized database should be established to log all reported/collected issues and to keep track of open and closed issues, referrals and time frames.
- **Reports and analysis:** a CFM system should be designed to easily produce reports and analytics needed by WHO. This could be to understand closure rates of issues or to review data to support with programme decision-making.
- **Data protection:** it is important that any CFM meets the UN *Personal data protection and privacy principles* (26). Make sure that the data protection focal point in the country office is involved in the CFM design or review process.

- **Confidential complaints:** affected people should be able to use the CFM to register a confidential complaint, to support the reporting of sensitive issues.
- **Assigning issues/referrals:** in a single agency or collective mechanism, the system should be designed to easily separate issues that need to be assigned and dealt with by different WHO teams, by specific organizations that are part of the collective mechanism, or by organizations outside the mechanism.
- **SOP:** an SOP should be developed that outlines roles and responsibilities, timelines, processes and actions for addressing and closing issues. It should also cover analysis and use of data from the CFM for programme decision-making, covering the full complaints and feedback loop in section 7.6 below.

7.6 The complaints and feedback loop

All CFMs should be designed to fulfil the feedback loop from the start. Therefore, all the following components must be planned for before establishing or joining a CFM.

- **Receive** issues from multiple channels.
- **Record** all issues received using a tool that is linked to a centralized database and data management system. The record should be date stamped and categorized using predefined categories. New categories of issues will emerge over time, and these should be added to tools.
- **Address** issues according to predefined processes for each complaint category. Some issues may be addressed on the spot by providing the needed information, whereas others may need to be referred to another team internally or externally. Set time frames within which each issue category should be addressed. Map out processes for addressing newly emerging categories as and when they arise.
- **Close** all issues once they are addressed. Log the date of closure in the data management system. This will enable identification of which issues are still pending and need addressing.
- **Analyse** complaints and feedback data regularly to identify trends in issues being raised in different locations and by different segments of the population.
- **Report** the analysed data into decision-making spaces, such as programme/operational meetings and senior management meetings. Reporting data to donors could also promote flexibility in how funding is used to meet changing needs.
- **Discuss** trends and outliers, as well as possible solutions.
- **Adapt** programmes and staff training in response to feedback.
- **Document** feedback trends, potential solutions and final decisions, as well as any feedback you were not able to respond to and why. Track the implementation of agreed adaptations.
- **Inform** communities about actions taken in response to their feedback, and where appropriate, involve them in designing these adaptations.

7.7 Sensitive complaints, including on SEA

- CFMs should be accessible, especially for women and girls and other vulnerable groups. Those receiving complaints should be trained in how to deal with sensitive complaints, including reports of SEA.
- Informed consent should be sought from the person making a report. They should understand how the report will be recorded, how the information they provide will be used, who and what will be shared with other people and for what purposes, and what can be kept confidential.

- Information shared during the report should be subject to strict information protection practices. Referral procedures and data storage should be safe and use encrypted files with strong password. A very limited number of people should be allowed access to information, and details of a report should be kept separately from personal identifying information.
- People reporting SEA issues should always have the option of doing so anonymously.

Box 3**Using complaints and feedback data in HeRAMS**

In contexts where there is a functional WHO, health or collective CFM, reports on barriers to accessing health facilities or lack of availability of medication or specific services, reported by affected people, is an additional information source that can feed into HeRAMS.

This will provide an additional community perspective on service availability, which can be used to triangulate data from other sources and contribute to the HeRAMS principle of openness. Complaints and feedback categories and data collection processes should be designed to ensure data collected is in a format that is useful to feed into HeRAMS.

Box 4**Early warning alert and response system (EWARS)**

EWARS is any surveillance system or component of a larger surveillance system that provides an early warning of acute public health events and connects this function to an immediate public health response. It is one of the most immediate and important functions of a surveillance system.

The EWAR global guidance (27) outlines the importance of using sources of data directly collected from affected communities, such as via hotlines or community reporters, where early rumours of an outbreak might be reported. WHO establishing or being part of a collective CFM will provide an opportunity for direct and live access to early warnings from communities.

This approach would also align with the HEPR commitment (4) to promote “community-driven detection, warning, surveillance, and response.”

EWAR: early warning alert and response; HEPR: health emergency preparedness, response and resilience.

7.8 Monitoring a CFM

- **Regularly assess the CFM closure rate** for incoming issues by analysing the difference in the logged dates when issues were received and the date of when issues were closed. Make sure that CFM SOPs include expected closure time frames for all issues, so that you know if you are meeting the standards you have set.
- **Ask affected people if they know about the CFM** and how to access it. Include a question about this in monitoring questionnaires on a regular basis.
- **Ask users of the CFM if they are satisfied with it** and if they received a response to their issue that was useful. Contact a sample of people who have reported issues to the CFM, and conduct a brief satisfaction survey to understand their experience.
- **Analyse CFM data to understand how accessible it is.** If the CFM collects data points on users such as gender, date of birth and geographical location, then this data can be used to analyse which groups are using the CFM and which are not. This will provide a starting point to investigate why some groups are using it less and how their access and comfort can be improved.

7.9 Further reading

Guidelines: interagency community-based complaints mechanisms, protection against sexual exploitation and abuse. Geneva: Inter-Agency Standing Committee; 2016 (<https://interagencystandingcommittee.org/sites/default/files/migrated/2021-03/Best%20Practice%20Guide%20Inter%20Agency%20Community%20Based%20Complaint%20Mechanisms.pdf>, accessed 27 November 2024).

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WFP guide to personal data protection and privacy: principles and operational standards for the protection of beneficiaries' personal data in WFP's programming. Rome: World Food Programme; 2016 (<https://docs.wfp.org/api/documents/e8d24e70cc11448383495caca154cb97/download/>, accessed 27 November 2024).

Chapter 8

Partnerships, collective AAP and coordination

8.1 Working with governments

Governments have the primary responsibility in emergencies. WHO's priority is to support a government in implementing its responsibility by collaborating closely with the ministry of health. AAP must be planned and implemented, as much as possible, with the cooperation and support of central and local government counterparts from the ministry of health.

- Link with any existing accountability mechanisms within the government or ministry of health, particularly those where people can raise grievances related to medical care, such as medical negligence. Look to strengthen and work with these mechanisms.
- Understand existing communication and awareness raising channels and protocols related to health, and make sure to include and strengthen these as part of your plans.
- Mainstream AAP into ongoing partnerships with the ministry of health, for example include it as part of planning discussions, in particular preparedness planning.
- Identify those within government that have responsibilities and capacity around communication and accountability and bring them into discussions.
- Prioritize data protection both in the support provided to governments and when sharing information with them.
- Include the ministry of health in your AAP and PSEA strategy or workplan.

There may be concerns from governments about establishing accountability mechanisms, especially where there is an active conflict or politically charged context. As far as possible, involve them in the design and take their concerns into consideration, including the avoidance of controversial terminology, while ensuring a strictly needs-based approach.

In situations where a government is actively oppressing its people, WHO should work independently of them, but together with other humanitarian actors, in implementing AAP activities, ensuring affected people know WHO is a neutral body. Where possible, involve local or de facto authorities. The strict implementation of data protection principles should be applied to all data collected from affected people, and opportunities to provide confidential opinions and feedback should be prioritized.

8.2 NGO implementing partners

WHO should encourage and support implementing partners to meet global AAP commitments, in particular local partners that may not have the resources to commit to specialist AAP positions. Partnering with local NGOs that represent and have closer connections to affected people will strengthen the AAP agenda as well as support WHO's work on localization.

Include AAP in any partner training to build knowledge and understanding.

Share WHO and other relevant AAP resources with partners, to support them with practical ideas and tools that they can use.

Encourage partners to attend any AAP coordination mechanisms or communities of practice, so they are part of response-wide AAP planning.

Appreciate that even where local, community-based partners do not have formal AAP processes in place, they are much closer to and better able to understand and represent the needs of affected people than international organizations.

Understand that partners may be at different stages of their AAP journey, and efforts to take the first steps to formalize AAP activities should be supported rather than seen as a barrier to working together. This is especially the case with smaller local partners with more limited resources.

AAP should be embedded in partnering processes to make responsibilities clear (see Table 11), but without putting in place prohibitive hurdles for local partners.

Table 11 Optimal AAP requirements when partnering with local partners

Partnering process	AAP requirement
Calls for proposals	Include a requirement to embed AAP approaches and activities across programmes.
Partnership review committee	Include a requirement to review partner AAP plans according to IASC commitments.
Partnership agreements	Proposals, budgets and logframes should include AAP activities and reflect the use of participatory approaches.
WHO monitoring of partners	Include follow-up on AAP activities during partner visits, reviewing evidence of plans and SOPs in place as well as data/information generated and how these have been used to adapt programming. Partner reports should also provide updates on AAP activities as well as analysis of AAP data and documentation of actions taken. Visits to partner programme sites should always include discussions with affected people and verification/monitoring of AAP activities from their perspectives.
WHO evaluations of partners	Include a review of AAP activities as part of partner evaluations. Include a requirement for affected people to be involved in planning and implementing the evaluation.

AAP: accountability to affected people; IASC: Inter-Agency Standing Committee; SOPs: standard operating procedures.

8.3 Collective AAP

Collective AAP is where all actors at a response level define and realize collective AAP outcomes by collaborating and harmonizing their implementation of AAP activities. Collective AAP reduces duplication and therefore minimizes potential confusion among affected people, by coordinating information streams from different agencies and access points for affected people who have a question, feedback or complaint. Collective AAP is guided by the IASC *Collective AAP framework (13)*, and WHO should support its implementation in the following ways.

- Participate in interagency AAP and PSEA working groups or communities of practice. This will make sure that WHO is part of the process of planning for collective, response-level AAP from the outset of a crisis.
- During planning processes, consider how AAP activities can be best coordinated and delivered together with other health actors and across sectors.
- Avoid implementing stand-alone data collection exercises that increase the burden on affected people.

8.4 AAP in health coordination

Mainstreaming AAP in coordination ensures humanitarian coordination mechanisms are accountable for maximizing results and protecting the rights of affected people.

The IASC *Standard terms of reference for humanitarian country teams (28)*, humanitarian coordinators and cluster/sector coordinators include responsibilities to embed AAP in the response. ‘Minimum commitments for participation in clusters’ (section 7 in the IASC’s *Guideline: cluster coordination at country level*) and the *Principles of partnership* (see further reading section 8.5) also reinforce the shared responsibility for cluster members to support AAP.

The Global Health Cluster has *AAP – operational guidance (6)* and *Minimum standards – AAP (7)* to guide the implementation of AAP in health clusters/sectors or outbreak coordination groups. That guidance provides overarching expectations that apply across all health partners, whereas this Handbook speaks directly to WHO processes and provides more detailed and prescriptive content.

8.5 Further reading

Leadership in humanitarian action: handbook for humanitarian coordinators. Geneva: Inter-Agency Standing Committee; 2024 (<https://interagencystandingcommittee.org/humanitarian-leadership-strengthening-section/leadership-humanitarian-action-handbook-un-resident-and-humanitarian-coordinator>, accessed 27 November 2024).

Guideline: cluster coordination at country level. Geneva: Inter-Agency Standing Committee; 2015 (https://interagencystandingcommittee.org/sites/default/files/migrated/2019-02/reference_module_for_custer_coordination_at_country_level_2015.pdf, accessed 27 November 2024).

IASC guidance on strengthening participation, representation and leadership of local and national actors in IASC humanitarian coordination mechanisms. Geneva: Inter-Agency Standing Committee; 2021 (<https://interagencystandingcommittee.org/operational-response/iasc-guidance-strengthening-participation-representation-and-leadership-local-and-national-actors>, accessed 27 November 2024).

Operational guidance, generic terms of reference for cluster coordinators at the country level. Geneva: Inter-Agency Standing Committee; 2013 (<https://interagencystandingcommittee.org/other/documents-public/operational-guidance-generic-terms-reference-cluster-coordinations-country>, accessed 27 November 2024).

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Annexes

Annex 1 Suggested AAP roles and responsibilities in the incident management team

The following AAP responsibilities should be embedded in incident management team job descriptions and form part of performance appraisal processes. Regional offices should lead this process by embedding AAP responsibilities in WHO country representatives' job descriptions.

Table A1.1 Incident management team AAP roles and responsibilities

Incident management team position	AAP responsibilities
WHO Country Representative	• Champion the voice of affected people in internal and external forums. • Hold ultimate responsibility for AAP in emergency operations. • Ensure appropriate repurposing of existing staff capacity on community engagement into AAP activities during an emergency. • Advocate for collective AAP approaches with UN coordination bodies (humanitarian country teams) and ensure WHO incident management team takes up collective AAP decisions made by the humanitarian country team. • Advocate for inclusion of AAP approaches in ministry of health emergency response activities. • Discuss WHO AAP approaches with donors and advocate for AAP funding. • Ensure external communications reflect WHO's people-centred approach in emergency response.
Incident Manager/WHE Coordinator	• Champion the voice of affected people in internal and external forums. • Integrate AAP measures and activities into response documents and plans (needs assessments, operational plans, results frameworks, workplans, humanitarian response plans etc.) as well as monitoring processes. • Include AAP as a standing agenda item for incident management meetings, to make space to listen and respond to the views of affected people. • Use data from AAP activities to understand the effectiveness of the response and to implement course correction as required. • Ensure that WHO clusters embed AAP in all the key functions and all the steps and tools of the humanitarian programme cycle. • Set AAP targets and regularly review progress. • Allocate resources for AAP and assign responsibilities, including an AAP focal point.

Incident management team position	AAP responsibilities
Designated AAP focal point (see Annex 2 for more detail)	• Establish internal AAP processes and coordination, provide technical guidance to partners, develop AAP workplan, ensure collation and use of AAP data to make programme decisions, and embed AAP in the work of WHO cluster. • Provide support and monitor progress on AAP in different programme areas, including in the cluster work. • Report to the Incident Manager on issues, challenges and bottlenecks.
Planning Officer	• Ensure AAP is represented in the different sections of the operational plan and participatory processes. • Ensure the involvement of affected people in programme planning.
Health Information Management and Risk Assessment Officer	• Include community structures of affected people as data sources in health information and risk mapping, ensuring these reflect marginalized populations. • Ensure AAP questions and participatory approaches are reflected in needs assessments. • Design health information systems, services and products in a way that maximizes AAP.
Data Management Officer	• Assist with analysis and mapping of complaints and feedback data, and with the use of graphics to present this information to decision-makers. • Reflect AAP activities in sitreps and donor reports.
Monitoring and Evaluation Officer	• Identify the most appropriate AAP indicator/s to include in the operational plan, donor logframes and implementing partner contracts. • Establish and track means of verification and related data collection tools and processes. • Analyse gaps in implementation and agree solutions with AAP focal point and other relevant teams. • Use complaints and feedback data as a source of monitoring data.
Health Cluster Coordinator	• Roll out the health cluster AAP operational guidance with support from the WHO AAP focal point.
Partner Coordination Officer	• Ensure appropriate WHO engagement in intersectoral AAP working group (when active). • Ensure WHO involvement in collective AAP approaches, such as collective CFMs. • Ensure WHO AAP commitments are reflected in partnership agreements, monitoring and reporting requirements.
Technical Officer Risk Communication	• Embed AAP messaging alongside risk communication messaging to the affected population. • Ensure messaging speaks to IASC standards as well as to the needs expressed by affected people.

Incident management team position	AAP responsibilities
Community Engagement Officer	Suggested AAP focal point (may be Risk Communication and Community Engagement Officer in some contexts). See AAP responsibilities under AAP focal point.
External Relations Communication Officer	Document WHO AAP good practice and results, and communicate these externally. Ensure the voices of affected people, in all their diversity, are reflected in external communication products.
External Relations Resource Mobilization Officer/Grants Management Officer	Understand donor commitments to and/or policies regarding AAP and share these to the Incident Manager and AAP focal point. Reflect AAP approaches in funding proposals and logframes, and in donor reports. Ensure WHO AAP commitments and actions are documented and reported to donors.
Field Manager	Ensure field-level implementation of AAP activities in coordination with the AAP focal point.

AAP: accountability to affected people; CFMs: complaints and feedback mechanisms; IASC: Inter-Agency Standing Committee; UN: United Nations.

Annex 2 AAP focal point terms of reference

Terms of reference

Country office AAP focal point/specialist

It is advised that the designated AAP focal point is under the planning and monitoring function of the incident management team.

Provide briefings and training to ensure staff and partners understand how to integrate AAP principles and approaches in their work

- Facilitate WHE briefings, training and staff inductions on AAP.
- Support emergency response teams to integrate AAP into their workplans and programme strategies.
- Maintain and provide staff with access to a depository of resources and tools on AAP.
- Provide AAP briefings to implementing partners.

Provide programme support to integrate AAP

- Support the country office to undertake AAP analysis, including preferred information and communication channels of affected people.
- Support the country office to formulate a strategy for communicating with affected people.
- Support the office to establish/join and effectively manage a CFM.
- Support the country office to integrate AAP into assessment, planning, monitoring, evaluation and reporting tools, including the use of participatory approaches.

- Support in monitoring progress on AAP in different programmes, including in the health cluster.
- Ensure collation and use of AAP data to make programme decisions.
- Support the country office to document AAP practices and results.

Coordinate the design and implementation of strategies to integrate and scale up AAP interventions and processes in country office emergency operations

- Ensure annual workplans include specific AAP interventions.
- Develop and oversee an AAP workplan and budget, assigning roles and responsibilities across functions and teams.
- Support leadership and teams to ensure AAP activities, data and updates are effectively reflected in country emergency programme processes, including:
 - emergency preparedness and response plans
 - monitoring frameworks
 - emergency programme meetings
 - NGO implementing partner cooperation agreements and partner reports
 - joint WHO/ministry of health emergency plans
 - sitreps
 - mid- and end-year reviews
 - country annual reports
 - donor proposals and reports.

Provide technical support to, and coordination of, collective AAP approaches and initiatives

- Participate in any interagency AAP/community engagement working group and ensure WHO provides input into collective AAP plans and approaches.
- Support the health cluster coordinator in implementing the health cluster AAP operational guidance and minimum standards.
- Support WHO in receiving and processing complaints and feedback referred by other agency mechanisms or any collective mechanism.
- Ensure feedback from affected populations is regularly reported and communicated in the health cluster.

Annex 3 AAP workplan template

Objective 1: Assign AAP responsibilities, support capacity-building and integrate AAP in key WHO emergency response processes and documents across the programme cycle and in donor proposals and reports.

Current status: *[Brief explanation of the current status to understand the starting point for this area of intervention]*

Table A3.1 Objective 1 activities

Activity	Expected output	Position responsible and collaboration	Time frame
<i>E.g. AAP focal point appointed and terms of reference agreed</i>	<i>AAP workplan developed and followed up on</i>	<i>Incident Manager, all ERF functions</i>	

<i>E.g. Give WHE staff AAP briefing during retreat</i>	<i>15 WHE staff with increased understanding of AAP responsibilities across functions</i>	<i>AAP focal point</i>
<i>E.g. Include AAP activities and indicators in review of operational plan</i>	<i>AAP embedded in response planning and implementation</i>	<i>AAP focal point & Planning Officer</i>
<i>E.g. Develop beneficiary monitoring questionnaire and implement quarterly, including monitoring of AAP activities</i>	<i>Data analysed and presented quarterly in operational management meeting, and minutes document planned actions</i>	<i>AAP focal point, Monitoring Officer & Incident Manager</i>
<i>E.g. Operational meetings include review, discussion and decisions on AAP data</i>	<i>Perspectives of affected people feed into programmatic decisions</i>	<i>AAP focal point, Incident Manager</i>

AAP: accountability to affected people.

Objective 2: Participatory processes are in place to ensure the meaningful engagement of affected people, in all their diversity, in decisions that affect the provision of WHO's emergency health assistance.

Current status: *[Brief explanation of the status quo to understand the starting point for this area of intervention]*

Table A3.2 Objective 2 activities

Activity	Expected outcome	Position responsible and collaboration	Time frame
<i>E.g. Undertake focus group discussions with affected people to feed into programme design</i>	<i>60 people from target communities involved in design consultation (women, elderly, people with specific needs etc.)</i>	<i>AAP focal point & Health Information Officer</i>	<i>Jan 2025</i>
<i>E.g. All programme teams to visit affected communities once per month</i>	<i>WHO staff regularly engage with affected people and have a better understanding of their needs, priorities and challenges</i>	<i>Incident Manager, programme teams</i>	
<i>E.g. Engage with local disability organization for assessment of health care accessibility</i>	<i>Recommendations for improved access to health care for people with disabilities</i>	<i>AAP focal point & Health Information Officer</i>	

Objective 3: Affected people are provided with key programme information, using channels that are appropriate and accessible.

Current status: [Brief explanation of the status quo to understand the starting point for this area of intervention]

Table A3.3 Objective 3 activities

Activity	Expected outcome	Position responsible and collaboration	Time frame
<i>E.g. Coordinate with PSEA working group and embed PSEA messaging across programmes</i>	<i>Affected people accessing health services have increased understanding of expected standards of staff behaviour and knowledge of how to register a complaint</i>	<i>AAP focal point & Technical Officer Risk Communication</i>	
<i>E.g. Develop a beneficiary-focused FAQ document, train staff and distribute</i>	<i>Consistent and accurate messaging being received by affected people</i>	<i>AAP focal point, Technical Officer Risk Communication & health workers</i>	
<i>E.g. Develop an AAP communication strategy</i>	<i>A plan is in place for delivering WHO's commitment to provide transparent information to affected people</i>	<i>AAP focal point & Technical Officer Risk Communication</i>	

AAP: accountability to affected people; FAQ: frequently asked questions; PSEA: protection from sexual exploitation and abuse.

Objective 4: WHO has an accessible system in place to receive and respond to complaints and feedback from affected people, and uses data to adapt programmes.

Current status: [Brief explanation of the status quo to understand the starting point for this area of intervention]

Table A3.4 Objective 4 activities

Activity	Expected outcome	Position responsible and collaboration	Time frame
<i>E.g. Map and document options and outline budget for joining collective CFM and present to senior management</i>	<i>Way forward for joining collective CFM identified</i>	<i>AAP focal point, Incident Manager & WHO Representative</i>	
<i>E.g. Develop and implement roll-out plan for joining collective CFM</i>	<i>WHO has CFM up and running</i>	<i>AAP focal point, Monitoring Officer</i>	

<i>E.g. Start receiving and responding to health referrals from existing CFMs run by other organizations</i>	<i>WHO starts receiving and responding to the health questions, feedback and complaints of affected people</i>	<i>AAP focal point & Monitoring Officer</i>
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AAP: accountability to affected people; CFM: complaints and feedback mechanism.

Annex 4 AAP in donor proposals

Given that AAP commitments and responsibilities are embedded in the humanitarian architecture, donors expect AAP approaches to be embedded in proposals for emergency funding. WHO should seek to reflect any of the AAP approaches and activities that are planned or already implemented in all emergency donor proposals. The below gives an outline of the type of information donors will expect to see.

Project rationale/justification/needs assessment/needs analysis

- Describe consultations undertaken with affected people to ensure their participation in identifying the priority needs to be addressed by the programme. This should include the number of people consulted, who was consulted (disaggregated by sex, age, disability and location), how these consultations were designed to be inclusive of the most marginalized, and a summary of their priority needs and inputs into programme design.
- Describe any other data sources used to understand needs that reflect the voices of affected people, for example, beneficiary monitoring data from previous programmes or complaints and feedback data. Provide numbers/percentages of people expressing certain needs disaggregated by sex, age, disability and location.
- Describe the differing needs of different groups of people, and back these up with data.
- Describe affected people's communication, information and participation preferences.

Project activities

- Describe how the programme design has been adapted to meet the expressed needs, priorities and preferences of affected people.
- Describe plans for the ongoing participation of affected people in decision-making throughout the programme and what tools and mechanisms will be used, for example, participatory monitoring, community committees, CFMs etc.
- Describe information provision activities planned, mechanisms to be used, key messages that will be shared and any adaptations that will be made to ensure these activities reach the most vulnerable people, including people with specific needs.
- Describe the CFM in place, adaptations made to ensure the CFM is accessible for vulnerable people and people with specific needs, and processes in place for issue referral and closure, including for sensitive complaints. Describe how complaints and feedback data will be used to feed into programme decision-making.

Annex 5 Beneficiary-focused AAP indicators

The indicators listed in Table A5.1 can be used within country office emergency operational plans, country office monitoring frameworks, donor logframes or implementing partner logframes. It is advisable to have at least one to two AAP indicators in all the above.

Use of the indicators requires the appropriate data collection mechanisms to be in place to measure a baseline and regular progress values. The data collection mechanisms can be implemented directly by WHO or by implementing partners.

Table A5.1 Beneficiary-focused AAP indicators

Topic	Indicator	Data collection mechanism
Satisfaction with programme intervention	% of target population satisfied that interventions addressed their priority health needs and concerns	HH/ community survey; Satisfaction survey at service point
	% of target population reporting barriers to accessing health services in the past week/month by type of barrier reported (Barrier examples: distance from health facility; transport costs; caregiving responsibilities; employment responsibilities; health issue not a priority within the family; safety/security concerns while travelling; language barrier; stigma around health issue; previous treatment by health centre staff)	Satisfaction survey at service point
Participation in and satisfaction with consultative processes	% of target population who have been part of a WHO consultative process	HH/ community survey; Satisfaction survey at service point
	% of target population who have been part of a WHO consultative process that feel their views and opinions have been taken into account and addressed	HH/ community survey; Satisfaction survey at service point
	% of the target population who feel their views have been taken into account in the assistance they are receiving	

Topic	Indicator	Data collection mechanism
Level of knowledge and understanding about programme interventions	% of target population able to identify key information on WHO health programmes (what/where/when/for whom)	HH/ community survey
	% of target population who feel well informed about the health assistance/services available	HH/ community survey
	% of target population who are aware of/understand the standards of conduct for WHO staff and partners, including PSEA commitments	HH/ community survey
	% of target population who know how and where to ask a question, make a complaint, or provide feedback on WHO programmes	HH/ community survey
Use of and satisfaction with complaints mechanism	% of target population who have used a WHO mechanism to ask a question, make a complaint or provide feedback	HH/ community survey
	% of target population who have used a WHO mechanism to ask a question, make a complaint or provide feedback and were satisfied with the response they received	HH/ community survey; Satisfaction survey at service point; Complaints and feedback monitoring
	% of issues raised by the target population which are responded to and closed within <i>[please insert time period appropriate to context and internal SOPs]</i>	Complaints and feedback data analysis

HH: household; PSEA: protection from sexual exploitation and abuse; SOPs: standard operating procedures.

Annex 6 AAP in the operational plan

Table A6.1 provides examples to use in an emergency operational plan of how WHO plans to implement AAP principles and commitments.

Table A6.1 AAP in the operational plan

Topic	What to include
Public health impact and immediate needs	Describe how consultations with affected people (including marginalized groups) have informed the development of the operational plan, and what adaptations or new activities resulted from these. Describe any particularly vulnerable or affected groups. State existing communication channels available to provide the affected population with information on WHO services and for the affected population to contact WHO with questions or issues. If there are gaps in these areas, then outline these.
Priority activities	Outline the communication strategy for targeting affected people, including the channels that will be used and how accessibility for different groups will be ensured. Outline any plans for ensuring the participation of affected people in decision-making across the programme cycle. Describe how WHO will receive, respond to and close complaints, feedback and questions related to planned response activities, including serious complaints (i.e. SEA, fraud and corruption).
Expected outputs and key performance indicators	Include AAP indicator/s to measure activities.

AAP: accountability to affected people; SEA: sexual exploitation and abuse.

Annex 7 Immediate AAP activities for a sudden onset emergency

The list of actions below is for use by country offices who may not have established any AAP activities and processes prior to the start of a crisis.

1. Assign and orient AAP/community engagement focal points for WHO and for the health cluster, with responsibility for promoting and leading all AAP/community engagement and accountability discussion and activities in the initial response, outlined in the points below.
2. Ensure community engagement and feedback is on the agenda of all operational meetings within WHO and the health cluster.
3. Instruct partners, including ministry of health, on minimum questions to determine community needs.
4. Collect, collate and share information from partner reports on community needs, that is, answers to the above questions, either in rapid needs assessment or through ad hoc key informant interviews with leaders of community organizations.
5. Coordinate and share information for communities (FAQ, service availability, contact information including any numbers to report SEA).
6. Be in touch with other UN agencies, international NGOs or AAP working groups regarding any existing or planned CFMs that will be used for the initial response, or any coordinated plans to establish one. Ensure WHO and health partner participation in collaborative CFM efforts.
7. Coordinate and implement initial community consultations through partners and ministry of health (ensure inclusive approach to cover needs of different groups).

Annex 8 AAP in PHSA

Table A8.1 describes the AAP information to be included in different sections of the PHSA document.

Note on source data:

- Where possible, use secondary data sources that have engaged with different vulnerable groups of affected people and reflect their perspectives on their immediate and long-term needs and priorities.
- These data sources could include complaints and feedback data from existing CFMs (WHO, partner or collective mechanisms); community consultation data; focus group discussion data; needs assessment; and monitoring or evaluation data that reflects the expressed views of different groups of affected people. Some qualitative data sources should be used.

Table A8.1 AAP in PHSA document

PHSA section	What to include
Health status and threats/ humanitarian profile	State what participatory data was used and which population groups' perspectives are represented. Identify vulnerable groups within the given context: groups that may be specifically vulnerable to certain health threats and groups that may be vulnerable as a result of societal norms, power structures or mobility issues that prevent them from accessing the health care they require (e.g. children, women and girls, people with disabilities, displaced and stateless people, refugees, elderly, religious or ethnic minorities, LGBTIQ+ people).
Health system status and local health system disruptions	This should consider the perspectives of affected people. For example, CFM data could be used to understand health system status and disruptions as reported by affected people. There may be disruptions that are best identified by affected people and communities, community organizations, community leaders and local and national actors.
Humanitarian health response	Participation and inclusion: reflect successful strategies and methodologies used by WHO, the ministry of health or health partners to involve communities in health-related decision-making, and the extent to which these have been successful at engaging the most marginalized or how they could be adapted to do so. Information provision and transparency: reflect any successful public health communication strategies already in place by partners, and the extent to which these have reached the most marginalized groups and could be used to communicate AAP and PSEA messages to affected people. Complaints and feedback: reflect information on existing CFM (including for PSEA), if these are accessible to the vulnerable and at-risk groups in need of assistance and can receive and refer health-related issues.
Information gaps and information source	Where any of the above information or data sources are missing from the PHSA, these should be identified as gaps in knowledge that need to be filled.

CFM: complaints and feedback mechanism; LGBTIQ+: lesbian, gay, bisexual, transgender, intersex or queer; PHSA: public health situation analysis; PSEA: protection from sexual exploitation and abuse.

Annex 9 AAP questions for needs assessment

Table A9.1 lists example needs assessment questions to elicit an understanding of affected people's health needs and priorities. The questions also help identify needs and preferences regarding information, participation, and complaints and feedback.

Table A9.1 Needs assessment AAP questions

Topic	Questions
People's priority health needs	• What are the most significant health needs facing you and your family at the moment? • How do you meet/deal with these health needs? • What are the most significant health needs facing your community at the moment?
People's priority health challenges	• Are you satisfied with the timeliness, relevance, appropriateness and quality of humanitarian health services and assistance? • What are the main challenges you experience in accessing the health services that you need? • Are the people most in need receiving health services and assistance? • Are you aware of people who may not be able to access available health services because of specific needs or an isolated location? Can you tell us more about them? What are their main barriers?
Types of assistance needed	• What health assistance do you and your family need? • How would you like to receive assistance? Do you have any preferences for the way services are provided? <i>[Especially important for frequently stigmatized services e.g. for gender-based violence survivors, mental health conditions, HIV etc., but also for those with mobility challenges]</i> <i>[Provide relevant examples/options for the context]</i>

Topic	Questions
Information and communication preferences	• What are the main channels of information available to you at the moment? • What channels of information are difficult for you to access and why? • What health information would you like to receive? Specify your top three priorities. • Where do you currently receive health information and information of available health services? • Who do you trust to provide you with accurate health information and information on available health services? [<i>Triangulate sources of information</i>] • How would you prefer to receive health information and information on available health services? [<i>Provide relevant examples/options for the context</i>] • Which language(s) would you like to receive information in? • Are you aware of anyone who may have difficulty accessing health information because of specific needs or an isolated location? Can you tell us more about them? What are their main barriers? • Do you know which organizations are providing health services in your area? • Do you know how you can contact those providing health care in your area to request support? • What would be the most convenient way for you to contact health providers to request help from them?
Feedback and complaints	• If you want to give your opinion to health care providers on the quality, accessibility and appropriateness of their services, do you know how to do this? • If you want to give your opinion to health care providers on the quality, accessibility and appropriateness of their services, how would you prefer to do this? [<i>Provide relevant examples/options for the context</i>] • How would you prefer to provide feedback to aid providers about the behaviour and conduct of their staff? [<i>Provide relevant examples/options for the context</i>] • In which language would you prefer to provide your opinion or register a complaint?

Topic	Questions
Participation	• Does anyone in your community have input into decisions about health care provision? If so, who? • Do you feel like you have a say in decisions that affect the health care provision in your community? If not, why not? • How important is it to you that you are involved in decisions about health care provision in your community? • How would you like health care providers to involve you in decisions about the services you will receive?

Annex 10 AAP in evaluations and JORs

Table A10.1 suggests AAP evaluation questions. All the questions are relevant for evaluations, but JORs should use questions under ‘relevance’, ‘effectiveness’ and ‘efficiency’ for monitoring purposes.

Table A10.1 AAP evaluation questions

Criterion	What the criterion means	Suggested AAP evaluation questions
Relevance	Was the programme/activity relevant to the needs and priorities of affected people?	• Were affected people, in all their diversity, consulted during needs assessments to understand their priorities? Did this include their preferred information, communication and participation requirements? • Was the programme designed to meet the needs that affected people expressed? • Did the design include adaptations required for particularly vulnerable groups?
Effectiveness	The extent to which the programme/activity achieved its results and objectives, including across different groups and locations	• Did affected people have access to the information they needed? • Did affected people participate in programme decisions that affected them? • Were affected people able to contact WHO to ask a question, raise an issue or provide feedback? Was WHO responsive to them at an individual and programme/activity level? • To what extent did feedback influence the effectiveness of the programme/activity?

Criterion	What the criterion means	Suggested AAP evaluation questions
Efficiency	Did the programme make the best use of resources to achieve its objectives in a timely way?	• To what extent did WHO work together with partners to share systems and resources in implementing AAP activities (i.e. use collective AAP approaches)? • To what extent did feedback influence the efficiency of the programme/activity?
Impact	What transformative difference did the programme/activity make, either positive or negative?	• Did the programme/activity have any enduring effect on people's well-being, human rights, gender equality or environment? • Were affected people in general, or any particular group, empowered by the programme?
Sustainability	The extent to which the benefits of the programme/activity will/are likely to continue	• Do affected people feel more prepared to tackle future health challenges than before the programme? • Are WHO local partners better able to implement programmes that are accountable to affected people?

AAP: accountability to affected people.

Annex 11 AAP activities at different stages of the AAP journey

Tables A11.1 to A11.3 show the different activities that should be implemented across the pillars of participation and inclusion, information and transparency, and complaints and feedback at different stages of a country office's implementation of AAP processes.

Table A11.1 Start of AAP journey

Pillar	AAP activities
Participation and inclusion	• Map existing community consultation platforms and methods being used for health programmes (within WHO, under the ministry of health or by other humanitarian health partners). Assess whether these ensure representation from the most vulnerable and isolated, whether they can be adapted to do so, or whether new platforms and methods are needed. • Speak to affected people to understand their needs and preferences when it comes to participating in decision-making on WHO's programmes. Understand their opinion on existing and possible new platforms and methods. Make sure this information is collected from the most vulnerable groups to plan for any adaptations to ensure accessibility. • Select the best platforms and methodologies for WHO to use to ensure the participation of affected people in decision-making. Decide how these will be implemented to overcome barriers to the involvement of vulnerable groups. • Map upcoming opportunities in programmes when key decisions will be made and when the participation of affected people is important. Schedule these in workplans. These should include opportunities across the programme cycle. Decide what level of participation you are aiming for in each of these opportunities. • Anticipate what organizational barriers may hinder the participatory process for each of these opportunities, and ensure solutions are considered at this stage. • Identify organizational decision-making spaces, such as operational meetings and senior management meetings, where the voices of affected people need to be reflected in decisions.

Pillar	AAP activities
Information and transparency	• Understand the communication context. • Map the existing communication channels used by WHO, the ministry of health or other health partners to provide information to affected people. Understand which are the most effective and whether they reach the most vulnerable and isolated. • Speak to affected people to understand their needs and preferences when it comes to the type of information they need and the way they would prefer to receive it. Understand their opinion on existing and possible new channels for providing information. Make sure this information is collected from the most vulnerable groups to plan for any adaptations to ensure accessibility. • Map opportunities to provide information to affected people that exist within programmes, for example where there is already face-to-face contact between affected people and WHO/partner staff or health workers, or where existing health messaging is being delivered and there is an opportunity to add to this. • Using the above, select the best communication channels for WHO to use to ensure information provision and transparency for affected people. • Decide on the areas of information that need to be detailed and delivered going forward.
Complaints and feedback	• Understand/map existing interagency or health-related CFMs. • Understand if they meet WHO's requirements. • Speak to affected people to understand their needs and preferences when it comes to providing feedback and making a complaint. Understand their opinion on existing and possible new channels for a CFM. Make sure this information is collected from the most vulnerable groups to plan for any adaptations to ensure accessibility. • Decide whether WHO will join an existing CFM, or if it is better for WHO to establish its own mechanism. • If establishing a new CFM, at a minimum WHO should be able to receive, process and close relevant health referrals from existing mechanisms.

AAP: accountability to affected people; CFM: complaints and feedback mechanism.

Table A11.2 Putting AAP systems in place

Pillar	AAP activities
Participation and inclusion	• Decide which participatory platform or methodology will be used for each identified opportunity for the input of affected people in decision-making. • Prepare to implement each participatory process by: – ensuring appropriate gender balance of staff undertaking these activities – training staff on participatory approaches – preparing key information for affected people in accessible formats – ensuring adaptations are in place to support the involvement of vulnerable groups – outlining questions needing answers or areas requiring input – deciding the best methodology for each opportunity. • Reassess organizational barriers for that area, and prepare for how these will be handled during the session. • Implement participatory processes and document affected people's views and perspectives. • Feed perspectives of affected people into programme decision-making spaces and processes. Ensure all perspectives are considered and addressed. If some ideas will be implemented, set a time frame for doing so. If some ideas cannot be implemented, document the reasons why. • Include information on adaptations to information provision and transparency activities made or not made, so that this is reported back to affected people. • Report on these processes, who and how many people participated and the resulting programme adaptations in WHO internal reports and donor reports. • Make sure that programme staff are regularly visiting affected communities to hear directly from them about their needs and experiences of WHO programmes and adaptations.

Pillar	AAP activities
Information and transparency	• Draft AAP messages under each of the previously determined areas of information. Start with an FAQ document directed at affected people to form the basis of information to be provided. This can be based on frequent questions coming from affected people, plus key information on WHO health programmes and their rights, including PSEA. The FAQ can be the basis for all beneficiary messaging and can be adapted to suit different methods of communication. • Translate the FAQ into local languages. Test the content and language with affected people for comprehension and to identify areas for improvement. Implement these improvements. Make sure to align terminology among health partners and develop a shared FAQ at health sector level if possible. • Train beneficiary-facing staff on the FAQ document and provide them with a copy. Make sure that this document forms the basis for verbal information provided to affected people to ensure accuracy and consistency. • Draft a communication plan that outlines the different channels and messaging to be used for each programme area. Make sure that adaptations are made to ensure communication reaches the most vulnerable and isolated. • Implement the communication plan.
Complaints and feedback	• Ensure all CFM requirements are in place before launching the CFM. • Launch the CFM through a community awareness campaign. This should be done even if WHO is joining an existing mechanism, to make sure WHO-targeted communities are aware of the CFM and how to access it. • Follow SOPs in place to ensure that the full feedback loop is addressed, including ensuring that all issues are closed on an individual level. • Keep a close eye on internal referral and closure processes to make sure that staff with these responsibilities are following SOPs. Make any adjustments needed to SOPs and roles and responsibilities. • Ensure that newly arising FAQs feed into updating the FAQ document. • Regularly analyse data to understand trends in different locations, and include unusual outliers. • Make sure this data is shared with operational teams and senior management in spaces where it can be discussed, and corrective action agreed, documented and followed up on.

AAP: accountability to affected people; CFM: complaints and feedback mechanism; FAQs: frequently asked questions; PSEA: protection from sexual exploitation and abuse; SOPs: standard operating procedures.

Table A11.3 Learning, adapting and improving your AAP

Pillar	AAP activities
Participation and inclusion	• Adapt participatory approaches to changing programmes and operational areas. • Monitor affected people's satisfaction with processes of participation. Understand how they think this could be improved and implement these improvements. • Monitor affected people's satisfaction with their level of involvement in programme decision-making and with the extent their views and opinions are being considered. Understand how they think this could be improved, and implement these improvements. • Assess the levels of participation of the most vulnerable groups across participatory approaches and understand if there are differences in their levels of satisfaction with these processes and their results. Understand how they think these could be improved and implement these improvements. • Review the level of participation WHO is aiming for across different programmatic decisions and plan to increase the level where feasible. • Review the extent to which the voices of affected people are feeding effectively into WHO decision-making spaces and if any changes or adaptations are needed to ensure that this improves.
Information and transparency	• Monitor affected people's knowledge and understanding of key programme information (e.g. how to access health services/ information, eligibility criteria, how to make a complaint etc.). • Assess the levels of knowledge and understanding of the most vulnerable groups and understand if there are differences for different groups. Understand how they think this could be improved in your communication strategy and implement these improvements. • Hear from affected people the best way to improve their knowledge and understanding and implement these changes. • Review and update the beneficiary-focused FAQ to reflect programme changes and to ensure transparency in information provided.
Complaints and feedback	• Monitor the CFM to understand coverage and accessibility, by disaggregating data by user demographics and including CFM monitoring questions in regular monitoring tools (or establishing new tools where no regular monitoring tools exist). Identify areas for improvement and understand perspectives of communities on how to make improvements. Implement these improvements. • Review the CFM closure rate and identify challenges and delays with processes. Identify and implement solutions/improvements. • Monitor the satisfaction of affected people with the CFM, and the way that issues are addressed. Identify areas for improvement and understand perspectives of communities on how to make improvements. Implement these improvements. • Review the extent to which CFM data is feeding effectively into WHO decision-making spaces and whether any changes or adaptations are needed to ensure this improves.

AAP: accountability to affected people; CFM: complaints and feedback mechanism.

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